

Maintenance Treatment Manual

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ABOUT THE HOPE MAINTENANCE TREATMENT PROGRAM

The HOPE maintenance treatment program is designed for individuals who have successfully completed the Preparatory and Core Curriculum. The maintenance curriculum is designed to help maintain treatment gains and to prepare the individual for life after probation or parole. As such, the following modules incorporate review of some of the more pertinent material from the Preparatory and Core Curriculum, as well as some new modules and exercises that build on previous material and prepare the client for the future.

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MODULE 1: Goal Setting and Good Lives Plan

Key Learning Points/Module Goals

- To review the main principles involved in goal setting
- To review the Good Lives Model and the implication to healthy living
- Provide direction on past and future goal attainment

Homework Assignments

- Homework: Wheel of Life (Homework 3.1.1)
- Homework: My Good Life Plan (Homework 3.1.2).

Handouts

- What is the Good Lives Model (Handout 1)

Goal Setting and the Good Lives Model

Facilitators should discuss the importance of goals with the group and link it back to the earlier goal setting exercise initially presented during the preparatory program and discussed throughout the core curriculum. Ask the group which goals they felt they achieved and are most proud of during their time at HOPE. Positive reinforcement is key. The purpose here is to orient the client toward the importance of acknowledging the goals that have been achieved as well as to establish a set of personal goals that will improve his or her life moving forward.

It will be important to remind clients, via discussion, about some key elements of goal setting and goal attainment. These include differences between short-term and long-term goals, avoidance versus approach motivated goals, and the SMART goal framework. Remember, clients often frame their goals as avoidance-based (i.e., things they should not do) such as avoid children and certain places. While this is appropriate and deserves some attention, considerable discussion needs to be placed on approach goals (i.e., things they will do in the future). Examples of approach goals are a) establish a healthy relationship and b) establish a group of prosocial friends.

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At this point in the curriculum, group members should have some understanding of these principles and have shown that they have applied them in their lives, at least to some extent.

Review of Good Lives Goals

Facilitators should provide the client handout: What is the Good Lives Model (Handout 1)

https://www.goodlivesmodel.com/General%20Description.pdf

Choose one group member at a time to read each section (e.g., What is the Good Lives Model, Common Life Goals, etc.) to the rest of the group. Encourage questions and feedback so that everyone is on the same page regarding what this model is all about.

Next, review the common life goals in detail. Considerable time should be spent reviewing each of these goals, their description, and common examples. Generate discussion and feedback while eliciting actual examples from clients pertaining to these goals. The key distinction here is to determine how clients have either 1) found positive ways to meet this goal and/or 2) found negative ways to meet this goal.

Common Life Goals (Yates & Prescott, 2011)

1. Health and Physical Safety- This goal is important to people who think about or spend time being healthy or making sure they can meet their basic needs.
    - Positive ways to meet this need: Exercising, eating well, taking care of physical self.
    - Negative ways to meet this need: Spending time looking out for threats to their well-being or safety; being hostile toward others seen as threatening.

2. Relationships and Friendships- This goal is important to people who think about or spend time with family, friends, or romantic partners.
    - Positive ways to meet this need: Spending time with family/friends, supporting family and friends, establishing or maintaining an intimate relationship.
    - Negative ways to meet this need: Being a member of a gang, being a member of a group such as NAMBLA.

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3. Peace of Mind- This goal is important to people who think about or spend time trying to be calm and relaxed or trying to reduce stress.
    - Positive ways to meet this need: Exercising or meditating to relax or stay calm, trying to reduce emotional distress or stress using adaptive coping methods.
    - Negative ways to meet this need: Using alcohol or drugs to relax or cope, using sex to cope.

4. Happiness and Pleasure- This goal is important for people who think about or spend time wanting to be content, happy, or to experience pleasure.
    - Positive ways to achieve this goal: Doing things that give them a sense of satisfaction or fulfillment, doing things that give them a sense of pleasure or purpose.
    - Negative ways to achieve this goal: Engaging in pleasurable but illegal activities, such as drug use or sexual abuse.

5. Being Good at Work and Play- This goal is important to people who think about or spend time trying to be good at work or leisure activities.
    - Positive ways to meet this goal: Being good at sports or hobbies, being good at a job, getting an education or training to become better at a job.
    - Negative ways to meet this goal: Stepping on others to get to the top, undermining another employee, sexually assaulting someone due to feeling jealous of her work accomplishments.

6. Learning or Having Knowledge in Areas that Interest Us- This goal is important to people who think about or spend time seeking knowledge or information.
    - Positive ways to meet this goal: Going to school, completing training, taking courses, learning about themselves in treatment, mentoring others.
    - Negative ways to meet this goal: Teaching children about sex via inappropriate activities.

7. Independence- This goal is important to people who think about or spend time being independent and making their own way in life.

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- Positive ways to meet this goal: Valuing personal choice in most situations, making plans to meet a goal, being assertive.
- Negative ways to meet this goal: Controlling, dominating, or abusing others, feeling that the world is a dangerous place or that they have to "get" others before they get them.

8. Having Meaning in Life- This goal is important to people who think about or spend time looking for a purpose or meaning in life.
    - Positive ways to meet this goal: Participating in religious or spiritual activities (e.g., church), mediation or prayer, reading or studying spiritual materials.
    - Negative ways to meet this goal: Interpreting one's own beliefs to support sexual offending.

9. Being Part of a Group- This goal is important to people who think about or spend time being a part of a specific group of people.
    - Positive ways to meet this goal: Being or wanting to be part of a group with those that have things in common (e.g., military), doing volunteer work, being an active member of their neighborhood.
    - Negative ways to meet this goal: Being a member in a gang.

10. Creativity- This goal is important to people who think about or spend time creating or trying new things or looking for excitement.
    - Positive ways to meet this goal: Trying new things they have not done before, painting, building, and drawing.
    - Negative ways to meet this goal: Trying newer types of sex or more exciting sex that crosses the line into abuse/non-consent.

Homework: Wheel of Life (Homework 3.1.1)

The therapist should hand out copies of the Wheel of Life (Homework 3.1.1). The analogy of the wheel is used to reflect the notion that a well-functioning wheel should be large and round. The smaller the wheel or more irregular it is, the harder it is to function properly. It is also good to note that most people don't have a perfect wheel, but that it is important to recognize the areas

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in need of improvement. The therapist should also indicate to the clients that the goal of treatment is to equip them with the skills and self-confidence in each area of the Good Life so that they will have a better and more satisfying life. Success in at least some of these areas will help ensure risk to re-offend is reduced.



Each individual is to assign a value from 0-10 indicating the degree to which he or she feels satisfied with their achievement in each of the nine areas of functioning. Participants are expected to present this to the group and be open to receive feedback and questions.

Facilitator Tip – Have the individuals first wheel of life, completed during the preparatory program, available. Present the differences to the participant and group and facilitate a discussion on such differences. Facilitators should highlight the development of these goals and how they relate to the Good Life Goals discussed. In-depth discussion on how the individual was successful (or not) in achieving his or her goals is helpful. Additionally, ensure that the individual is clear about the goals that are a priority as well as next steps; remember to utilize the SMART (specific, measurable, achievable, relevant, and timely) goal framework.

My Good Life Plan

At this point in treatment, clients should be familiar with the components of goal setting and the Good Lives Model and have some practice attempting to implement these principles in their road to rehabilitation. This assignment will be integral to formalizing their plans for the future and will provide some direction on steps they can take in this last phase of treatment to achieve their good life. A proper Good Life Plan has two unique but related goals:

1. The development and implementation of a comprehensive “map for living” that includes all ingredients to a good life and;
2. The identification of strategies needed to respond to problematic situations that may disrupt or threaten the client’s functioning in other areas of his or her life.

Facilitator Tip – Be aware of potential problem situations as they arise. Indeed, we all have problems that arise, but when they do, some individuals are tempted to return to offending or

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they may experience deviant thoughts. Potential problems include stable and acute risk factors, such as self-regulation problems, relationship conflict, and emotional distress, to name a few.

Homework: My Good Life Plan (Homework 3.1.2)

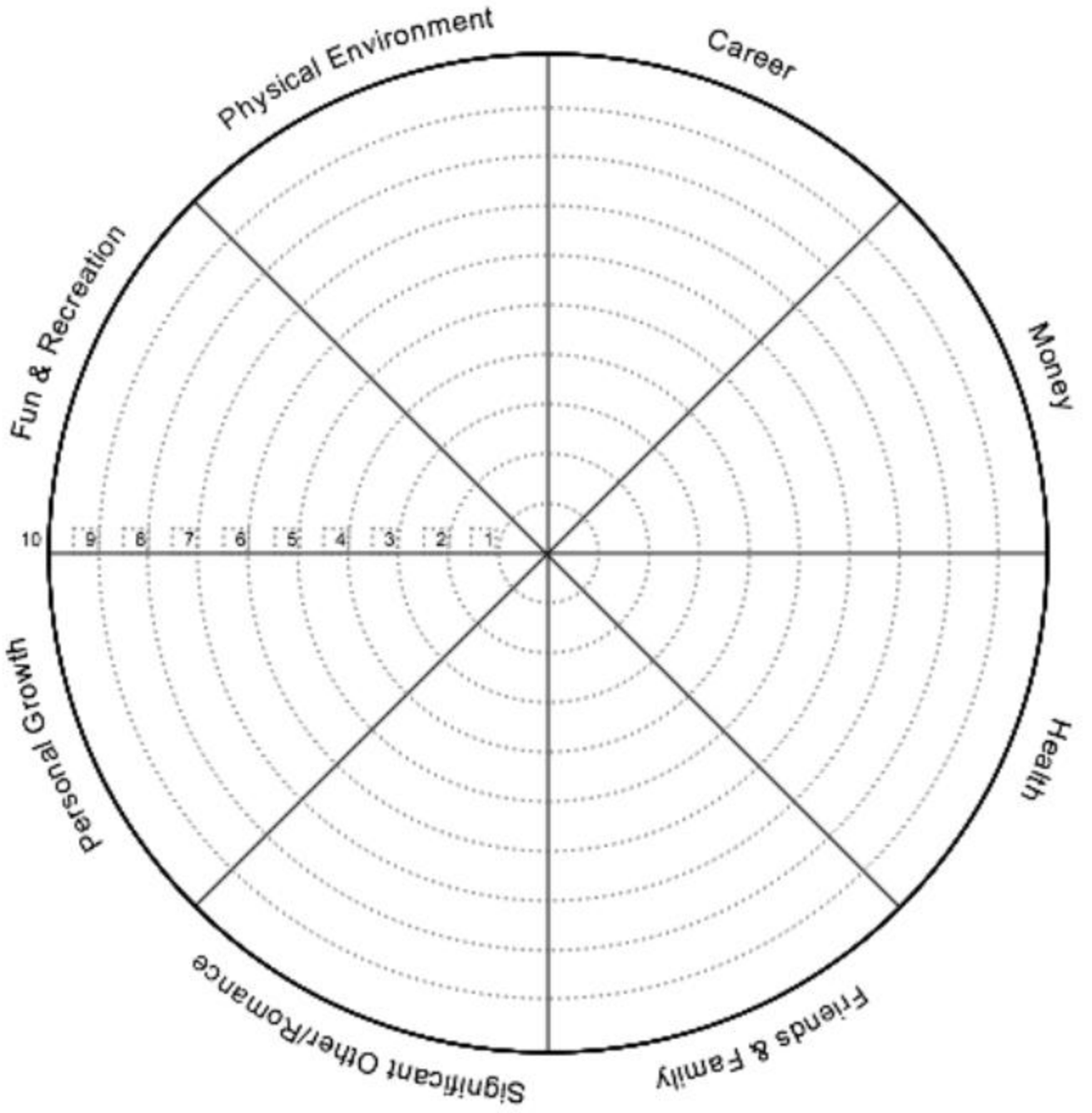
Each individual must complete the table below and be prepared to present it to the group. Most individuals will prioritize some goals over others (e.g., high, moderate, or low). The purpose of this assignment is to ensure that the actual group member presenting the assignment is clear on the goals that he or she prioritizes, how he/she defines this particular goal, his/her plan for achieving it, and any potential obstacles. Group feedback and questions are required to fully flesh out the contents of this assignment.

Sample client exercise would appear as follows:

Goal	What I want	How will I get it	Obstacles
Inner Peace High Moderate Low	Free from inner turmoil and less anger	Meditate for 20 minutes every day Continue with my thought record	High stress job Conflict with co-workers

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Homework: Wheel of Life (Homework 3.1.1)



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Homework: My Good Life Plan (Homework 3.1.2)

Goal	What I want	How will I achieve it	Potential Obstacles
Health and Physical Safety Low Moderate High			
Relationships and Friendships Low Moderate High			
Peace of Mind Low Moderate High			
Happiness/Pleasure Low Moderate High			

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Being Good at Work and Play Low Moderate High			
Learning and Knowledge Low Moderate High			
Independence Low Moderate High			
Meaning Low Moderate High			

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Being Part of Group Low Moderate High			
Creativity Low Moderate High			
Other: Low Moderate High			
Other: Low Moderate High			

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What is the Good Lives Model (Handout 1)

The Good Lives Model

What is the Good Lives Model?

The Good Lives Model recognizes that all human beings have similar goals in life, and that offending relates to attempts to meet these life goals. However, offending as a means to achieve life goals results in harm to other people, as well as problems in life. The Good Lives Model helps people understand the relationship between their life goals and offending, and how to achieve life goals and live a satisfying life without harming others.

Common life goals

Everyone's idea of what makes up a good life is different, however there is a set of life goals that are common to most people. The importance that people place on each goal helps define what matters most in their lives. These goals include:

- Health and physical safety
- Relationships and friendships
- Peace of mind
- Happiness/pleasure
- Having meaning in life
- Being part of a group
- Creativity
- Being good at what we do – either in work, hobbies, or both
- Learning and having knowledge in areas that interest us
- Independence (i.e., being in control of our lives)

As human beings, we all seek some or all of these goals in our lives. But sometimes, we encounter problems in our attempts to achieve these goals.

Problems experiencing our good life

Seeking out life goals can be like finding our way through a maze – sometimes we encounter dead ends and challenges. Instead of trying to navigate our way through the maze it can be tempting to take short-cuts to get what we want – including using drugs, doing crime, and being violent. While these short-cuts might give us some immediate satisfaction – they also give us many long-term negatives that take us farther away from our goals and our good life. For example, someone who is having relationship problems might turn to violence in a desperate attempt (i.e., a short-cut) to hold onto a relationship. That moment might bring a very brief sense of satisfaction, however the long-term consequences – for the offender and the victim – are far from satisfying.



How can the Good Lives Model help me or someone I care about?

The more short-cuts we take, the more lost in the maze we become. Psychologists, social workers, probation officers and others can help people to navigate their way through the maze by addressing some of the problems that led them to take short-cuts. Likewise, friends and family can help by showing their loved ones other ways to get what is important in their lives that do not cause harm to others and that do not lead to problems in their lives. The Good Lives Model helps people identify what they want their life to look like – and how to get there without hurting others.

Available at www.goodlivesmodel.com

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MODULE 2: Mood Management and Coping

Key Learning Points/Module Goals

- Review core components of emotions
- Discuss and practice key emotional regulation techniques

Homework Assignments

- Mood Monitoring and Thought Record (Homework 3.2.1)
- Radical Acceptance (Homework 3.2.2)

Handouts

- The Three Minds (Handout 2)

Understanding Emotional States

A brief review of emotions is helpful and facilitators should ask the group members to define emotions. Emotions have been defined as a "process, a particular kind of automatic appraisal influenced by our evolutionary and personal past, in which we sense that something important to our welfare is occurring and a set of psychological changes and emotional behaviors begins to deal with the situation" (Ekman, 1992).

Facilitator tip: Although much of this content could be delivered in a psychoeducational fashion, it is strongly recommended that the information is delivered with the goal of having an in-depth discussion. Very little (if any) of the content in this manual should be read directly to the client but rather discussed and related to their personal experiences.

While emotions can vary in **duration**, they are typically brief, and occur in response to particular events, situations, or memories. Emotions generally occur along a spectrum from **pleasant** (e.g., joy, excitement, and love) to **unpleasant** (e.g., depression, anger and anxiety), and can also differ in **intensity** (e.g., weak, mild, strong, intense).

Remind the clients that humans experience **7 BASIC OR UNIVERSAL EMOTIONS,** regardless of language, regional, cultural, or ethnic differences. These emotions include:

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1. Happiness
2. Sadness/Anguish
3. Anger
4. Fear
5. Surprise
6. Disgust
7. Contempt

The feeling wheel is another useful optional handout to provide again (they should have received it in Block I). Ask one member of the group to present on the feeling wheel to the other group members.

At minimum, group members should be reminded that the feeling wheel differentiates the relatively core or basic emotions and expands to include more 'colorful' emotional experiences. Similarly, the discussion should reiterate the concept of basic versus complex emotional states. *The facilitator should ask the group members how they differentiate basic from complex emotional states*. Essentially, some emotions are considered secondary to the more primary basic or universal emotions discussed earlier. For example, resentment can result from anger, hopelessness from sadness, and embarrassment from guilt or shame.

Next, the facilitator should review the three component model of emotions. Given that this is the maintenance program, groups members should present with at least a foundational knowledge of these key principles. It is suggested that the facilitator first ask the group members to list the three components. After these are presented, the group members should be able to 1) define each component and 2) provide examples from their specific life circumstances.

The **Three Component Model of Emotion** (Barlow et al., 2011) was developed as a framework for examining and describing emotions to help identify and describe an emotional experience. These three components include:

1. Physiological
2. Cognitive

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3. Behavioral

Physiological

The Physiological component pertains to the statement: **"How I feel in my body"** and relates to the physical responses associated with specific emotional states. These are usually autonomic responses that occur without much exposure. Some of the most common are associated with heart rate and breathing. Below are some common examples of different types of physical symptoms associated with just a few emotions:

Anger –

- Increased heart rate
- Increased breathing
- Sweating, sensation of heat in the head and/or body
- Upset stomach
- Shaking

Fear –

- Increased heart rate
- Increased breathing
- Shaking
- Difficulty sitting still
- Increase in acute arousal of the five major senses

Joyfulness –

- Increased heart rate
- Smiling
- Laughing
- Relaxed muscles
- Even breathing

Sadness/Depression –

- Lack of energy
- Slow movement
- Crying
- Upset stomach
- Shoulders hunched

Everyone is unique in their physical appearance and in their emotional responses. Being angry doesn't mean that you will have the symptoms listed above. Some people, when scared, will laugh or giggle. People respond differently, so it is important for you to learn what physical symptoms are related to your emotions. You may notice that some of the emotions share the same physical symptoms. Therefore, it is equally as important to identify the cause and purpose of your emotional response in order to put it into the appropriate context. This will be further discussed in the following section.

Cognitive

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The Cognitive Component pertains to the statement: **"What I think."** Thoughts are often associated directly with feelings, and are a useful indicator for recognizing emotional states. Common questions clients should ask themselves and which have been repeated throughout this program is, "What types of thoughts do you notice when you feel depressed or angry." What about when you feel happy?

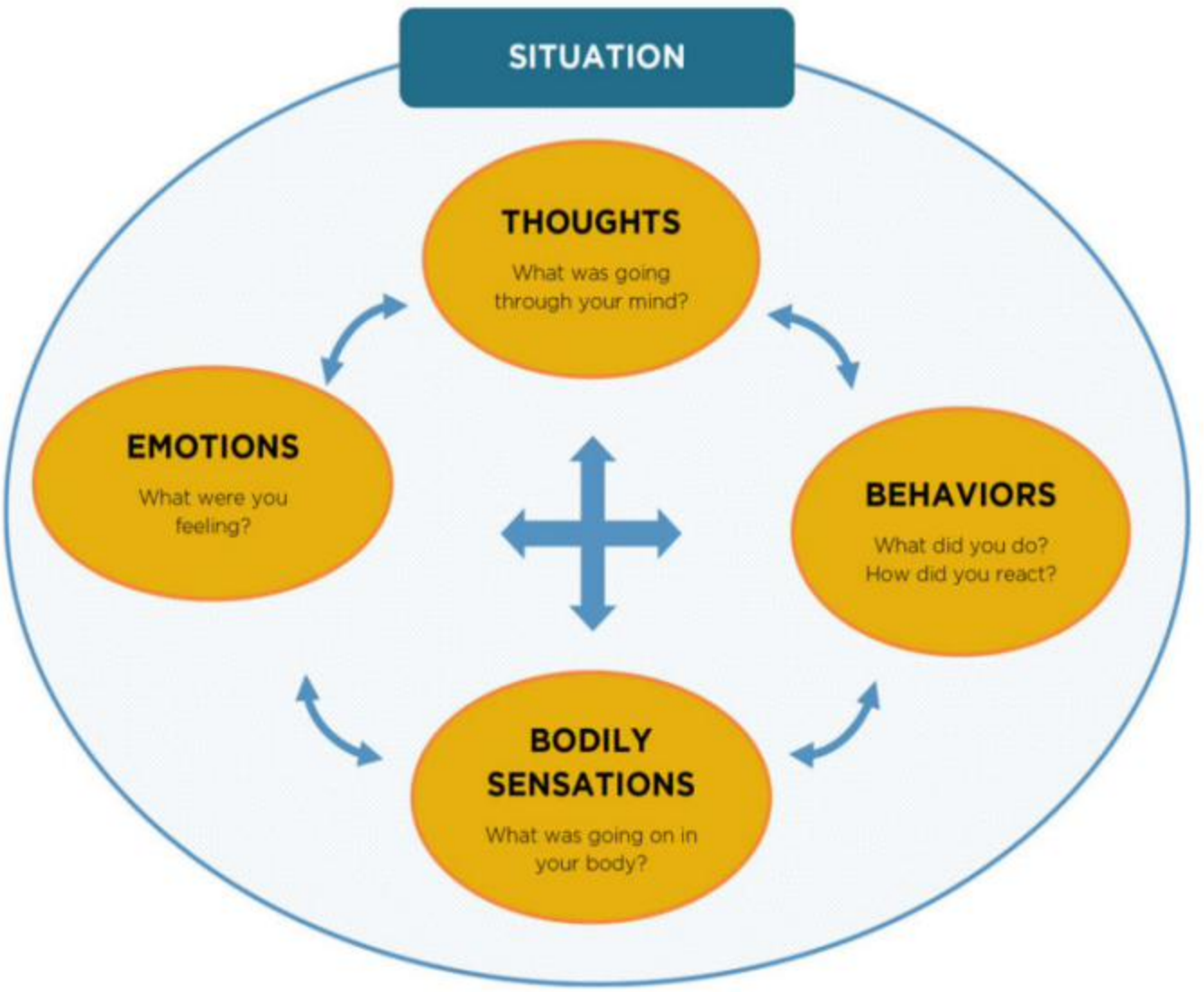
Thoughts and emotions are mental constructs, but can be differentiated by attaching thoughts to statements that begin with "I think", and emotions to those that begin with "I feel." It is not uncommon for individuals to use "I feel" statements with thoughts, such as "I feel that I shouldn't have to discuss my emotions." In reality what they are saying is, "I do not *think* I should have to discuss my emotions because I am *feeling* anxious."

Behavioral

The Behavioral Component refers to **"What I do."** This pertains to actions or behaviors as a response to emotional states. People can respond to feelings without thinking because our bodies often respond impulsively in order to deal with the situation. Barlow and colleagues (2004) have referred to these as **Emotion-Driven Behaviors** (EDBs).

At this point, it would be helpful to remind participants about Cognitive Behavioral Therapy (CBT) and the key components to cognitive restructuring. *Cognitive Behavioral Therapy* emphasizes the notion that changing maladaptive thinking leads to changes in mood and behavior. Therapists use CBT techniques to help individuals challenge their thinking patterns and beliefs and replace "errors in thinking," such as overgeneralizing and catastrophizing, with more realistic and adaptive thoughts, thus decreasing emotional distress and self-defeating behavior. These errors in thinking are known as cognitive distortions.

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When using CBT, or related approaches, clients should understand the importance of thoughts on our emotions. Many people understand this premise by learning the ABC model of psychological functioning and change. The ABC model states that it is not typically an *activating event* (A) that contributes to problematic emotional and behavioral *consequences* (C), but rather it is what people *believe* (B) or say to themselves about the *activating event*. *The event itself (A)* can be either an external situation or a thought or other kind of internal event, and it can refer to an event in the past, present, or future.

The following homework exercise (Homework: 3.2.1) is an essential exercise/tool to effectively self-monitor emotional states and ultimately become more skilled at regulating one's own

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emotions. Hopefully, clients have continued using this framework since completing the core program and have been presenting on it during check-ins. Nevertheless, it is a valuable tool to re-introduce here so clients can present on essential CBT elements during check-ins and other pertinent points during this phase of the program.

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Homework: Mood Monitoring and Thought Record (Homework 3.2.1)

Monitoring your mood is one of the most essential skills you will learn in order to become a master at regulating your emotions. Clients are to carefully monitor their moods during the following weeks and report back to the group using the homework assignment below.

Prompting Event: What is the event that triggered this?	Interpretation: What I said to myself?	Emotion and Intensity (0-10 scale)	Bodily Sensations

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Coping: Emotion Regulation

At this point in the program, group members should present with a solid understanding of emotional states and be very skilled at recognizing these states within themselves. Emotional recognition is a crucial part of healthy living and also contributes to effective emotional self-regulation.

Generally speaking, emotional regulation defines the ability to exert some degree of control over our emotional state, which also has an influence over behavior. Most people understand and discuss emotion regulation in the context of “down-regulation” such as when we attempt to reduce the intensity of emotional states. It does, however, also incorporate “up-regulation” such as when there is imminent danger or a challenge and we need to amp up our emotional states such as excitement or anxiety.

Psychologist James Gross has noted that people can act to control their emotions at different points in time, such as before the emotion has taken effect (i.e., antecedent-focused emotion regulation) and/or after they have experienced the emotion (i.e., response-focused emotion regulation).

There are a few categories or types of emotion regulation. The first is **suppression** which is a response-focused strategy that involves inhibiting or suppressing an emotional experience after it has been activated. In contrast, **reappraisal** is an antecedent-focused (i.e., before the emotion has been activated) strategy that involves interpreting one’s situation in a more adaptive and accurate manner. Other strategies include acceptance, mindfulness techniques, etc., several of which will be reviewed in this module

Facilitator Tip – Ask the group members to select which broad type of emotion regulation technique is best: suppression or reappraisal. Elicit examples from all participants about times they used one versus the other and the effect (and affect) they experienced. After some discussion, it is helpful to point out that, statistically, studies have shown that suppression results in less positive emotions, worse relationships, and reduced quality of life compared to those who reappraise or utilize another adaptive strategy.

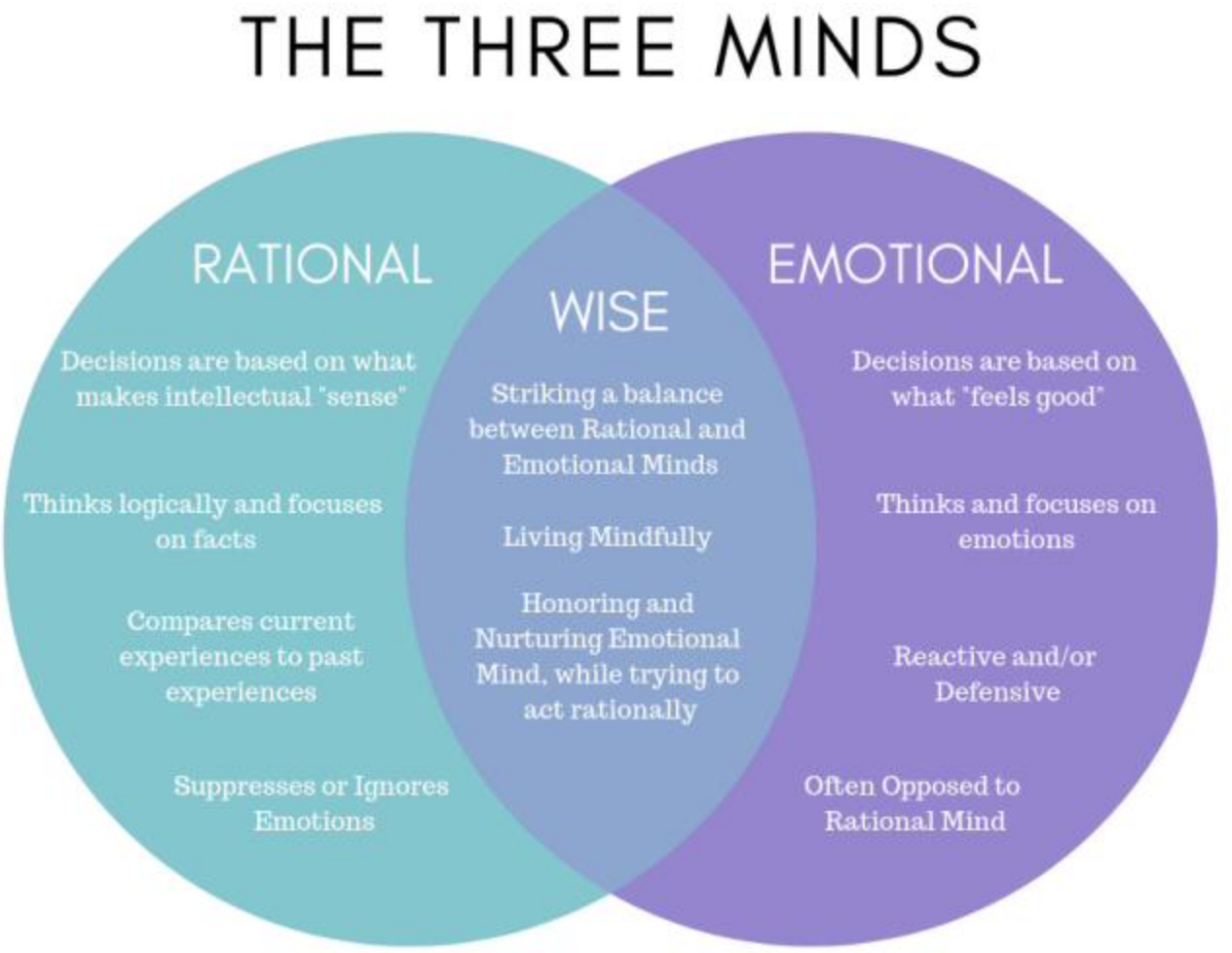
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Adaptive Skills to Regulate Emotions

Given the importance of DBT skills, particularly with regard to emotion regulation capabilities, it would be important to review the “three minds” discussed in the core program and provide the handout (Handout 2) to members again if they no longer have a copy. Spend time reviewing the core concepts and elicit thoughts from the group members.

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Handout 2: The Three Minds



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The goal, of course, is to live our life in "wise mind," although this is not always possible no matter how skilled one is. However, to help ensure we live our life in balance more often than not, there are a set of skills that can be practiced. Linehan (1993) referred to these skills as "core mindfulness skills" and were broken down into the WHAT and HOW skills.

Facilitator Tip – It is helpful to ask group members to define and describe these concepts prior to going into much detail to determine how much information was retained from the core program. Of course, detailed descriptions should then be provided to ensure proper understanding and skills should be practiced along with feedback and coaching.

The WHAT skills

- **Observe** – This skill asks you to be "in the moment" and surrender to the experience from a distance by allowing all thoughts and emotions to happen without reacting, as well as taking note of all sensory information (sight, sound, feel, smell, taste).
- **Describe** – With this skill you will define your experience by using words to explain what thoughts and feelings you experienced. The important caveat of this skill is to resist the desire to attach meaning to the thought or feeling. Simply describing the experience is the primary goal here.
- **Participate** – As in the observe skill, you will surrender to the experience, but instead of watching from a distance, you will immerse yourself in the experience and learn to go with the flow of events. Attend only to what is necessary in the moment and appropriate to the situation. Think of this as adjusting the volume of your voice to fit a particular environment so you can be heard. The final part of participating is *practicing*. As you practice, your response will become more natural.

The HOW Skills

The *HOW* skills can be viewed as adjectives and adverbs for the *WHAT* skills. They provide information about the manner in which the *WHAT* skills are to be performed. They are non-judgmental, one-minded, and effective.

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- **Non-Judgement** – Let the experience and the thoughts and emotions related to it just "be." Resist the temptation to "escape" the experience, although it may not feel completely comfortable. Reject nothing. *What you resist, persists.*
- **One-Minded** – Attend to the moment. Though your mind may wander (e.g., I wonder what I'll have for dinner), maintain focus on the experience that is presently happening, pulling yourself back in when you become distracted
- **Effective** – As with the other skills both in HOW and WHAT, this is concerned with being "in the moment" and allowing the experience and your responses to happen without concern with whether it is right or wrong. With mindfulness, the virtue or value is in the outcome of experience, not the correctness. Again, focus on the present and not a "wishful" or "could be" response and result.



Experiential Exercise: Wise Mind Meditation

Remind group members about the difference in emotional and rational minds and the benefits to adopting "wise mind." At its most basic form, wise mind helps us make healthy decisions about our life. A useful tool that can reinforce adoption of wise mind is to practice meditation. Some people can be turned off by meditation given its occasional spiritual undertones, but it does not have to involve spirituality (although it can). Meditation is essentially a skill to be practiced where we train our minds to be present, in the moment, and less reactive. However, like when building a muscle, one practice session won't change anything; it is best to incorporate this practice into your daily life, ideally when not overly stressed or anxious. Even 10-minutes per day can make a huge difference. Research has shown that meditation has a positive influence on anxiety and tension, improved sleep, chronic pain, improved blood pressure, as well as other benefits.

There are two basic forms of meditation that can be practiced daily (i.e., Progressive Muscle Relaxation and Deep Breathing) and have shown to have an important effect on healthy living. It is recommended that clinicians practice both types of relaxation techniques with clients in group and to encourage clients to continue to practice these strategies daily. Returning to these exercises in group is also recommended on a regular basis.

1. **Progressive Muscle Relaxation**

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Facilitators, provide a brief description of progressive muscle relaxation to members and indicate that you will be practicing this as a group. Progressive muscle relaxation is a technique that was developed in the 1920's based on the notion that physical relaxation can facilitate mental relaxation. The idea is based on research showing that the process of tensing and relaxing muscles can promote relaxation and increases skill in noticing muscle tension.

There are many scripts available to read to the group if that is desirable to practice progressive muscle relaxation. Alternatively, you can play the following YouTube clip and follow along together or bring in any number of apps (e.g., CALM, Headspace) that can help facilitate this exercise with the group.

https://www.youtube.com/watch?v=Z95gPdFC7GM

2. **Deep Breathing**

Deep breathing is another method that can be very helpful for relaxation, especially when practiced consistently. One of the benefits of deep breathing is that it can be practiced virtually anywhere at any time.

It is recommended that clinicians play the following video and have all group members participate. Again, other alternatives (e.g., CALM, Headspace) are acceptable.

https://www.youtube.com/watch?v=bvdzTs0m510

Radical Acceptance

Radical acceptance is another important aspect in developing a wise mind and is also a key distress tolerance skill (described below). As noted by Linehan (2015), radical acceptance is a complete and total acceptance of the facts of reality. It involves acknowledging facts that are true and letting go of a fight with reality. **Acceptance is not approval or being against change, but accepting what is currently true.**

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It is important to present the concept of radical acceptance and then generate a discussion on the concept and application to real life examples. Many individuals struggle with the idea of radical acceptance and the difficulty in application is well documented.

Most people have, on some occasions, likely said “I can’t stand this” or “This isn’t fair.” Ask the group members for examples of this reaction or way of thinking. The fact that the group members are on parole and mandated to attend treatment is often provided as an example.

Some important points to discuss include:

- Accepting reality is difficult when life is painful.
- Accepting reality doesn’t mean agreeing that it is okay.
- Tough experiences are part of life.
- When one attempts to avoid or resist reality, you simply add suffering to an already difficult situation. You create more misery.

Acceptance means that you transition from your resistant thoughts into thoughts that are more conducive to living a good life. An example would be: “I’m in this situation, and I don’t approve of it, but it is my reality in this moment and I can’t change it.” Of course, we can change our response to the reality, regardless of how uncomfortable it is, which is what radical acceptance helps with.

Homework: Radical Acceptance (Homework 3.2.2)

Participants are to describe at least three events where they have said “why me” or “this should not have happened.” Have them discuss ways they could see this situation differently, keeping in mind a radical acceptance perspective. Would this perspective have changed their emotional state in the moment? Elicit feedback and questions from the group after each presentation.

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Homework: Radical Acceptance (Homework 3.2.2)

Describe at least three events in your life where you said “why me” or “this should not have happened.” Essentially, the times where you attempted to a resist the reality. Discuss ways you could have approached this situation differently given your knowledge of Radical Acceptance.

Event # 1:

Event # 2:

Event # 3:

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Distress Tolerance

Sometimes it is difficult to find a quiet space to engage in a mindfulness meditation exercise. Sometimes we need to sound the alarm and have a quicker response to a distressing emotional response. Of course, the goal is not to completely eliminate the emotion, but rather lower the intensity just enough for some space to refocus and refresh (Linehan, 1993).

Considerable time can be spent on the following skills. Facilitators are encouraged to ensure that clients 1) have a comprehensive understanding of each skill and 2) have determined which skills are more effective for them. Group exercises and role-plays can be very effective for practicing some of these skills; between-session "behavioral experiments" where clients practice implementing these, recording the results, and discussing during the following group are particularly important.

Wise Mind ACCEPTS (Linehan, 1993) – The purpose of this skill is to give potential options for distracting from a particularly distressing event, and potentially provide an opportunity to intervene and alter the emotional response.

ACCEPTS is an acronym where each letter has a potential option.

A – *Activities*: This means getting involved in something other than attending to the emotional experience. Choices may include exercising, walking, read a book, listen to music, etc. It is important to be specific about what you intend to do. What exact song, album, or playlist will you listen to? What type and how many push-ups will you do?

C – *Contribute*: This type of activity focuses on helping others. Choosing to contribute might mean volunteering your time, writing a note to a significant other, cooking dinner for the family, buying or making a small gift, or a random act of kindness for the person behind you in the drive-thru.

C – *Comparisons*: Generally, comparing yourself to others can be counterproductive, especially if we're only doing it to downgrade our own life. However, making comparisons to other situations can help take the sting out of our present circumstances.

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E – *Emotions*: This is a process of finding an activity that will help exchange the current emotion for one less intense and distressing. An example of this would be listening to a high tempo, upbeat, song when feeling depressed or extremely sad.

P – *Pushing Away*: This option involves rejecting the current emotion in the present moment and denying it any energy. Statements such as, "I am choosing to not let the anger get the best of me," and "Whatever" are examples of this. A word of caution: Pushing away should only be used sparingly and only in extreme cases. Emotional experience is important, as previously noted, otherwise persistent use of this skill can be counterproductive.

T – *Thoughts*: Here the intervention is quite simply changing the thought to one that is more positive or distracting. It can be relevant or completely unrelated to the present situation. The intention here is to lessen the fuel provided to the emotional response.

S – *Sensation*: Instead of distracting activities, using this intervention involves distracting a thought by experiencing a sensation. This could be clenching your fists as hard as possible and counting to 10, followed by releasing them, and then repeating the process a couple of times. Another could be eating something extremely sour or holding a piece ice in your hand. Sensation is another device that should be used in small doses (Linehan, 1993).

IMPROVE the Moment (Linehan, 1993)***:*** Improving the moment is concerned with instantaneously changing the tone of the present situation to reduce the intensity of emotional reactivity. Like Wise Mind ACCEPTS, this is an acronym for potential options to choose from to address the current situation.

I – *Imagery:* This skill involves using your imagination and visualizing a different situation from the one in which you find yourself. While it is not a physical escape, it can provide some relief in the moment. The important component here is to make sure that the image you envision is not dangerous or provokes anxiety.

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M – Meaning: You have likely heard the expression, "everything happens for a reason." This is a similar construct encouraging you to find the purpose and meaning of what you are experiencing. Perhaps it is an opportunity to grow stronger or even simply learn more about a particular emotion or thought.

P – *Prayer:* This type of prayer is concerned with surrendering to the given experience and connecting with the distressing emotions. Prayer in this context is not requesting deliverance or seeking clarification. The goal is focusing and gaining increased understanding.

R – *Relaxation:* Due to the connection between mind and body, one way to approach regulating emotion is to relax the body. This can be achieved through progressive muscle relaxation or controlled breathing exercises.

O – *One Thing in the Moment:* This intervention is essentially one of the components of the HOW skill discussed above. It involves attending to only one issue, thought, or emotion at a time.

V – *Vacation:* Generally speaking, this can be taken literally. In essence, you choose to absolve yourself of any responsibility for as little as a few moments to no more than 24 hours. The focus is on ***you*** and your well-being, and nothing else. It does not mean being apathetic or inactive, but participating in something that brings you some solace.

E – *Encouragement:* This skill is marked by the use of positive self-affirmations (i.e., I am awesome, I can do this, I have mad skills, etc.). An alternative way to practice encouragement is to imagine how you might help someone close to you who is experiencing the same situation. Think of the types of positive statements you might impart to them (Linehan, 1993).

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While many other interventions out there exist, the ones above are a good foundation on which to develop other skills. Be mindful, however, of the reality that you need to practice them when you are ***not*** experiencing significant distress. In addition, time is required to become both comfortable and skilled at implementing these strategies. Choose a few to focus on and serve as the tools in your Swiss Army emotional regulation toolbox. Trying to internalize too many skills is the last thing you want to do, because it is an odd paradox to feel overwhelmed and stressed about using emotion regulation/distress tolerance skills. Finally, be patient and enjoy the process! As highlighted above, emotion is the force behind our thoughts and also adds color to our lives.

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MODULE 3: Cognitive Beliefs and Schema

Key Learning Points/Module Goals

- To enhance understanding of belief systems and explore the impact on emotions and behaviors.
- To solidify an understanding of cognitive distortions and how they have played a role in sexual offending.
- To increase the ability to challenge beliefs in a way that is more aligned with the evidence of a situation.

Homework Assignments

- Homework: My Schemas (Homework 3.3.1)
- Homework: Cognitive Distortions (Homework 3.3.2)

Handouts

- Three Components of Beliefs (Handout 3)

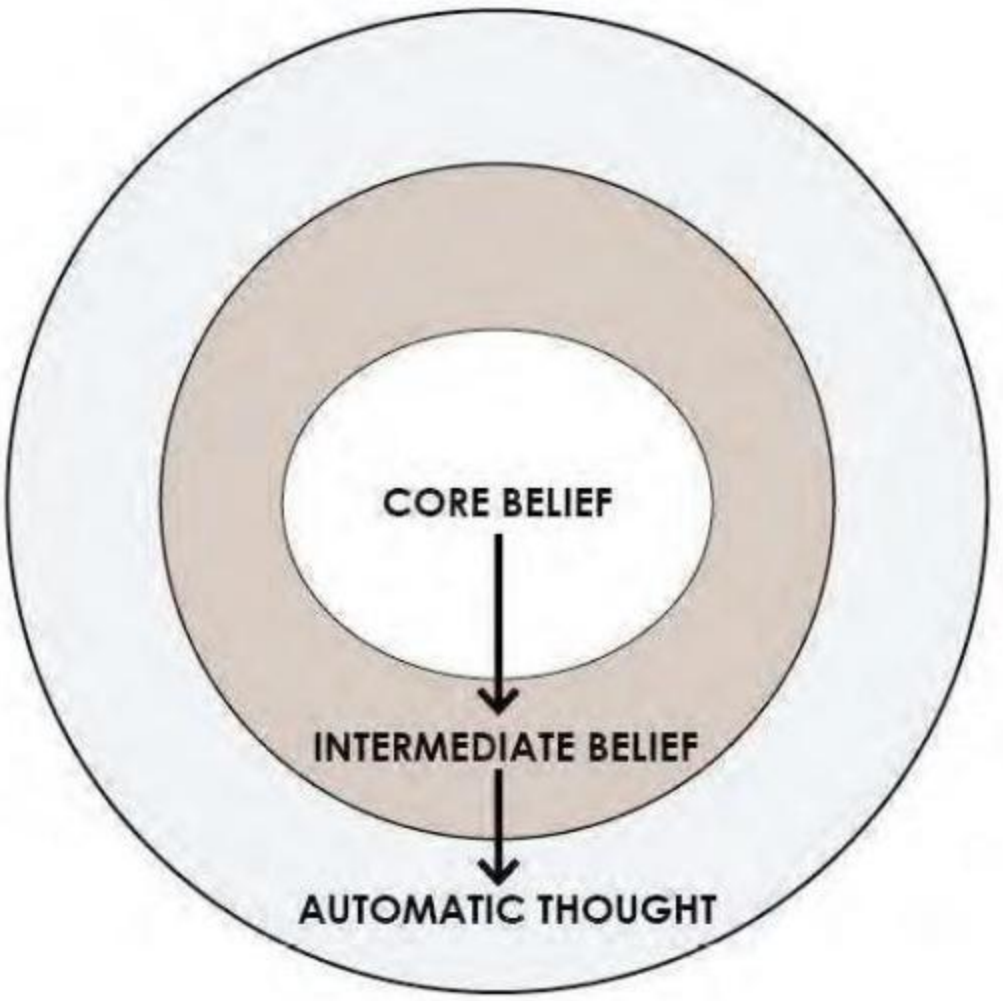
Our cognitive beliefs; that is, what we say to ourselves, give us permission to commit crimes and engage in behaviors that are not consistent with who we see ourselves as being. Group members should recall that people respond to similar situations in a very different way based, in large part, on the differences in how they interpret the situation. Our beliefs and beliefs system affects our thoughts, emotions, and our behaviors. Our thoughts are at the root of all behavior. As such, it is important to build on the clients' understanding of how his/her personal belief system developed and how it contributed to sexual offending behavior as well as other prior bad decisions.

It is helpful to review Handout 3 with clients: Three Components of Beliefs. There are three components to a belief system: *automatic thoughts, intermediate beliefs, and core beliefs.*

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Three Components of Beliefs (Handout 3)



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Automatic Thought (AT): Automatic thoughts are fixed beliefs and thoughts that you have every day. Everyone has them. They are separate self-statements or ideas that occur so rapidly that it doesn't seem as if they are even conscious thoughts. They seem to come out of the blue for no apparent reason. Automatic thoughts are typically specific in content and tell you something directly about the present situation you are experiencing. This has also been defined as a type of cognitive appraisal in that it helps filter our experiences and allows us to respond to situations in a particular way. They also occur continually throughout the day as we move about and engage in constant evaluation of our environment and ourselves. Again, automatic thoughts set the stage for how you will interpret and/or respond to a given situation; whether that is influencing another thought, eliciting an emotion, or driving a behavior. Even though we may be unaware of the source of the thought, we usually have a familiarity with the emotions we experience.

Facilitator tip: It is important to ask the clients to define, describe, and relate these definitions to their own life and understanding prior to providing a more explicit description.

Automatic thoughts have both benefits and drawbacks. A benefit is that they help you process information quickly and efficiently. If you had to process every thought you have for every new and unique circumstance, you would likely not get much done. They also have protective factors that keep you safe from threats, and quite possibly save your life. Take for example when you are driving and approach a traffic light at an intersection that is changing from green to red. Chances are you automatically begin to brake and slow to a stop. That is, unless you try to beat the yellow light! As you brake and come to a stop, you are seldom aware of the chain of thoughts that cause you to apply pressure to the brake pedal. Examples of statements might include, "red means stop," "if I run the light I'll get a ticket," and so on. If you had to consciously process any one of these statements before you applied the brake, you would be through the intersection before you had a chance to respond, putting yourself and others at risk.

As mentioned, some automatic thoughts are not beneficial and may result in negative consequences or results. Think back to the previous module where you were asked to list all the thoughts you had when you were told by your supervisor/significant other/probation officer, "I need to talk to you." What were those thoughts? Were any of them negative? Examples might

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include, “Oh no! What did I do now,” “I’m in trouble,” and so forth. These types of thoughts might have put you on the defensive, including creating fear, anger, or worry; and the more you had these thoughts, the more intense the emotions became. Then, you might be tempted, or driven to engage in behaviors that result in negative outcomes. These might include avoiding the conversation, lying, using substances, hiding, etc. As you can see, automatic thoughts can often keep us in behavior patterns that are self-destructive and harmful to others.

Intermediate beliefs are attitudes or rules that a person follows in his/her life that typically apply across situations (not situation specific as with automatic thoughts). They make up the second layer of belief systems. Intermediate beliefs can often be stated as conditional rules: “If x, then y.” For example, “If I have money, then I will be loved by others.” Individuals create these assumptions by categorizing the information they receive from the world around them. These rules guide thoughts and subsequently influence behaviors. They may also be described as expectations. Dysfunctional core beliefs drive dysfunctional rules (intermediate beliefs) and in kind, automatic thoughts. For example, the belief “I am unlovable” may be driving the conditional rule, “if I am rich, then I will be loved by others,” which may drive obsessive thinking about ways in which to get rich quick and engaging in illegal behavior.

Core beliefs lie at the center of the belief system and consist of the thoughts and assumptions we have about our environment and our experience of it. They are most often formed in childhood and are influenced by many factors, including our families, cultural and ethnic backgrounds, and, may also be shaped by other social and temporal factors (e.g., socio-economic status, the time period during which we were born, etc.). Core beliefs are solidified over time as a result of our ongoing perceptions and interpretations of experiences. Aaron Beck, the “father” of Cognitive Therapy, suggested that core beliefs tend to involve either acceptance (“I’m unlovable”) or achievement (“I’m incompetent”) issues. Core beliefs may also be about others (i.e., stereotypes). Because individuals encountering personal/psychological challenges tend to store information consistent with negative beliefs, but ignore evidence that contradicts them, core beliefs tend to be rigid and pervasive. Core beliefs tend to be more global and cut across multiple life and environmental domains. For instance, if the core belief is, “I’m no good/unlikeable/unworthy,” then the individual may feel that way in their family, their job, or their group of friends.

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Core beliefs lay the groundwork for both intermediate beliefs and automatic thoughts. Because they are most frequently instilled in us early in life, we are not often able to recognize them at all. Hence, they contribute to the instantaneous automatic thoughts we have; and, the rule and value systems (intermediate beliefs) that we endorse and live by in our daily lives. If you stop, think, and question the source of your beliefs, you might simply say, “I’ve just always thought that way” or “That’s what I was raised to believe.”

Cognitive Schemas

As discussed throughout this curriculum, core beliefs, or cognitive schema as some people call them, are a set of beliefs and attitudes that help people understand and organize what is going on around them. Jeffrey Young, one of the leading figures in schema therapy and research, defined schemas as, “any broad organizing principle for making sense of one’s life experience” (Young et al., 2003, p. 7). Many schemas are formed early on in life and become entrenched as we continue to encounter life experiences. Schemas can be positive and adaptive or they can be negative and maladaptive.

People with different schemas can perceive events in their world very differently than someone with a different schema, and this can also influence behavior in very different ways. For example, a person with a schema that he is not a good person will interact with the world in a much different way than someone with a schema that they are good and trustworthy. Another example involves authority figures.

Ask the group how person A would respond in any number of scenarios (emotionally and behaviorally) versus person B

Persona A has been pulled over by the police and believes police to be hostile and punitive

Person B has been pulled over by the police and believes police to be inherently trustworthy and that they do their best to make the community safer.

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Types of Schemas

A few schemas have been identified that have been found to be more common among people who have committed sexual offenses.

1. **Children as Sexual Objects**. This is the belief that children are able to consent to sex in the same way that adults can and that children enjoy sex. This schema can also allow those who think this way to believe that it is okay for adults to "teach" children about sex through direct sexual experience. Because of this schema, some people may tell themselves that it is okay to have sex with kids because they are teaching them (a cognitive distortion).

2. **Entitlement**. According to this schema, one believes that it is okay to have sex with whomever a person wants and that everyone deserves to have sex. The belief is that one can do whatever he or she wants.

3. **Dangerous World**. Someone with this schema believes the world is violent and uncontrollable (i.e., a dangerous place). The schema involves the idea that others are out to get you and that one can obtain benefit and satisfaction in any way possible.

4. **Personality and Sexuality are Uncontrollable**. According to this schema, people were born a certain way or became a certain way and they are not going to change, so even attempting to change is pointless. In this schema, the same principles apply to sexuality: "This is who I am." The person who holds this schema often views committing sexual abuse as behavior that is outside of his control. He believes it is okay to act on his sexual desires, even if the other person doesn't or can't consent to sex.

5. **Sexual Activity Without Other Violence is not Harmful**. This schema is the belief that sexual behavior is only abusive when it involves clear acts of intentional physical violence. This schema leads people to commit sexual abuse that is still harmful but in

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ways other than through violence. People with this schema often do not recognize the other types of harm that sexual abuse causes, such as psychological harm.

Homework: My Schemas (Homework 3.3.1)

Following the introductory discussion on schemas, groups members should respond to the following questions (Yates & Prescott, 2011) and be prepared to present it to the group. Group feedback and discussion should follow.

1. What are some schemas you have used in your life?

2. How do your own schemas affect your interpretations of events or other people?

3. How do you own schemas affect your reactions to events or other people?

4. Think about the mental shortcuts you have used during your life. Which of the specific schemas or shortcuts above do you think you might have used? Please explain

5. What kind of schemas do you think might have been involved in your sexual offending?

Cognitive Distortions

Cognitive Schema are considered a small subset of cognitive distortions. Considerable time was spent in the core program on cognitive distortions. Given the importance of thinking errors in emotions and behaviors, it is helpful to review with the group again.

Ask the group members to define what cognitive distortions are and why people use them. Cognitive distortions refer to the assessments and judgements that people make in a situation as it is happening. Cognitive distortions are the things that people tell themselves in a situation to make themselves feel okay about their reactions and behaviors.

Present the following cognitive distortions. Since this is the maintenance phase, individuals should be able to present with a relatively good understanding of what each distortion describes and provide examples from their own life.

1. **All or Nothing Thinking** – Also referred to as "black and white" thinking, "polarized thinking," or "splitting." The belief that things are all one way or the other, either all good or all bad. People are either friends or enemies, either complete successes or total failures, either saints or monsters. This distortion ignores the complexity of situations and individuals and reduces everything down to a single judgmental quality.

   Example: "I didn't do everything perfectly, so I'm a total screw-up." Or "That guy said something that made me mad, he must be a complete asshole."

2. **Catastrophizing** – Grossly exaggerating the potential negative consequences of a situation. Any bad thing that happens becomes the end of the world, and you can't think about anything else or keep things in perspective. This can result in "doomsday thinking," when you become so focused on the possibility of a terrible crisis that you become paralyzed and cannot respond to a negative situation with any positive solutions.

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Example: “My boss said I made a mistake – now I’m going to be fired and I’ll get evicted because I can’t pay my rent, I should just quit before that happens.” Or “That person said they didn’t want to go out with me – I must be ugly and I’ll never be with someone, I shouldn’t even try.”

3. **Mind Reading** – Believing that you know what someone is thinking based on their behavior, or based on what you expect them to be thinking. There is also the reverse of this, assuming that other people know what you are thinking without you telling them. This distortion ignores the fact that a guess is not the same thing as knowing, even if that guess is partially correct, and also ignores the fact that the human mind is complex and often contains more than one thing at a time.

   Example: “I saw that dirty look you gave me; you must hate me.” Or “My wife shouldn’t have gotten mad when I called her that name; she should have known I was joking and I didn’t mean anything by it.”

4. **Fortune Telling** – Believing that you know in advance what will happen in a situation, or that you know how things would have turned out if someone had done something differently. Just like Mind Reading, this distortion ignores the fact that a guess is not the same thing as knowing; we can all guess a specific outcome once in a while, but there’s a big difference between guessing an outcome and knowing the future. This distortion ignores the fact that anyone can be wrong, even when they’re sure they’re right.

   Example: “Every time I mention this to someone they get mad; therefore, everyone I mention it to in the future will always get mad.” Or “This only happened because my co-worker screwed up; if they had just stayed out of it, everything would have been fine.”

5. **Personalization** – Believing that you are the cause of events which may have nothing to do with you, or believing that you were the target of events which may have not been directed at you specifically. This distortion ignores the fact that we are not the center of the universe, no matter how much we may feel that we are. There can be a number of

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reasons why something happens or why someone does something, and just because we were there or were affected by it doesn't mean that we were the cause.

Example: "My mother is angry; therefore, I must have done something to make her angry." Or "The teacher yelled at the whole class, but I know they were really yelling at me."

6. **Focusing on the Negative** – Only paying attention to things that affect us negatively. Also called "invalidating the positive," because when you are so focused on bad things, you may forget to notice the positive events, or may believe that they are unimportant. This distortion ignores the fact that situations often have both positive and negative elements at the same time and that positive things are just as important as negative ones.

   Example: "When I made that speech, I got some words wrong and some people laughed. Sure, they all clapped at the end, but I'm sure I looked like an idiot because I made a mistake." Or "Every relationship I've ever been in had ended badly for me. Sure, there may have been some good times too, but in the end, relationships are nothing but pain."

7. **Denial/Minimization** – Believing that certain things are tiny or unimportant when, in fact, they may be quite important. This often takes the form of comparing things to bigger things so that they seem smaller by comparison. This distortion ignores the fact that little things often seem big to others, just as things that seem big to us often seem little to others. Each person has their own experience of what is important and what is not, and this distortion ignores the fact that each of those experiences is valid.

   Example: "I only hit my son once; some people hit their kids all the time. Therefore, what I did wasn't so bad." Or "I feel proud of what I've accomplished, but no one besides me cares. I guess compared to what others have accomplished it's not a big deal after all."

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8. **Should Statements** – Focusing on the difference between the way the world is and the way you want it to be, even when it is not within your power to change the world. Besides, "Should," these statements also take the form of "Must," "Ought to," and "Have to" statements. This distortion ignores the fact that it is not up to us to decide how the world "should" be and makes us waste our time focusing on a problem that we cannot solve instead of directing our energies towards problems we can solve.

   Example: "When I do everything right, I should get rewarded; it's not fair that I am punished when I don't deserve it." Or "That isn't the way I would have handled the situation; that person should have handled it the way I would have."

9. **Emotional Reasoning** – Believing that your negative emotions reflect a negative reality; literally, that if you *believe* something bad is true, therefore it *must* be true. Like Mind Reading, this distortion ignores the fact that anyone can be wrong even when they're certain they're right. This distortion is particularly difficult to deal with because often our negative feelings really do feel true, even when there is no evidence to support them other than the feeling itself. This distortion also ignores the fact that negative feelings will change with time, and that no negative feeling lasts forever.

   Example: "I feel hopeless, like nothing will ever work out right. That must be true, nothing will ever work out right, so why even try?" Or "I feel afraid that people don't like me, and I worry that they think I'm stupid and ugly. That must be true they don't like me and I am stupid and ugly."

10. **Immediate Gratification** – Believing that if you want something, you have to fill that need right away, and you cannot let a need go unfulfilled, as if you were at the mercy of your needs without any control over them. This can also manifest as a desire to end negative emotional states right away without considering the consequences. This distortion ignores the fact that most needs don't actually have to be filled right away. This distortion also ignores the fact that trying to immediately fill a need often comes with negative consequences that could have been avoided by showing self-restraint.

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Example: “I was horny and wanted to have sex, so I couldn’t stop myself even when she said no.” Or “I got upset by something he said, so I had to end the conversation and walk out. I couldn’t stand being upset for another second.”

11. **Unrealistic Expectations** – Believing that the only solutions to problems are perfect ones, and that these solutions solve the problem for all time. We may expect ourselves to enact these perfect solutions or we may expect them to be enacted by others for our benefit, but we can’t be satisfied with any solution that is less than perfect. This distortion ignores the fact that no solution is ever perfect, and even problems that have been solved tend to reoccur over time. This distortion puts us in danger of being frustrated with ourselves because we can’t live up to our own expectations, or disappointed when others don’t live up to those expectations.

    Example: “I can’t get married until I find a partner who will never get angry with me or criticize me for anything.” Or “They told me the medication would help my symptoms but I still have some problems, so I guess the medication is worthless.”

12. **Victim Stance** – Believing that if you were hurt, someone must have done it on purpose and the fact that you were hurt means no one else’s feelings matter. This often manifests as believing other people were in control of things you do or were responsible for your actions. This distortion ignores the fact that, even when we are hurt, we are still responsible for our own actions, and just because someone hurts us doesn’t mean they were doing it deliberately. Like Emotional Reasoning, this distortion is often difficult to deal with because when we are hurt or frightened it is often difficult to think about anything else. However, the fact remains that other peoples’ feelings are as important to them as ours are to us, and we cannot ignore them even when we are upset.

    Examples: “What she said really hurt my feelings, so I had to hit her; she was trying to get me mad and she deserved what she got.” Or “I don’t see why anyone else is complaining since I’m the one who was hurt by this. They were the ones responsible, not me, so what do they have to complain about?”

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13. **Justifying** – Believing that if there is a good reason for doing something, that negates all the reasons for *not* doing that thing. This can also manifest as "explaining away" the negative consequences of an action by focusing on the intent of the action. This distortion ignores the fact that the intent behind an action does not necessarily dictate the outcome. Just like someone can hurt us without meaning to so can we have a perfectly good reason for doing something that has negative consequences for ourselves or others and what we intended doesn't necessarily make the outcome any different.

    Examples: "Yeah, I was driving drunk, but my friend really needed a ride and she didn't have any other way to get home." Or "I was stealing to feed my family. No one can blame a man for doing whatever it takes to look out for his loved ones."

14. **Desire for Fairness** – Believing that the world needs to be fair, and any time it isn't, that justifies acts of revenge or retribution. This is really a combination of Should Thinking, Unrealistic Expectations, and Justifying, and can contain elements of Personalization or Immediate Gratification as well. This often manifests as feelings of being hurt by something unfair and feeling entitled to "balance the scales," either by hurting someone else or taking something to make up for the hurt feelings. This distortion ignores the fact that everyone gets hurt sometimes and everyone feels that they have been treated unfairly sometimes. The world does not come with a set of rules that say that everything has to seem perfectly balanced to everyone at all times. Sometimes we have to accept something that doesn't seem fair instead of setting out to get vengeance or settle the score.

    Examples: "I didn't do anything wrong, but I got punished anyway, so if I go down, everyone who punished me is going down too." Or "I grew up without the advantages wealthier people had, so if I steal from them, I am just getting back what I'm owed."

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Homework: Cognitive Distortions (Homework 3.3.2)

Have the clients list and describe the *Cognitive Distortions* that 1) played a role in their sexual offense and 2) impacted other areas of their life. They also should include whether or not they challenged these distortions and, if so, how.

1. The thinking errors that played a role in my sexual offending behavior included:

2. The thinking errors that played a role in other bad decisions I have made included:

3. If I was back in these situations again, how would I perceive them now?

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Homework: My Schemas (Homework 3.3.1)

Please answer the following questions and be prepared to present it to the group.

1. What are some schemas you have used in your life?

2. How do your own schemas affect your interpretations of events or other people?

3. How do you own schemas affect your reactions to events or other people?

4. Think about the mental shortcuts you have used during your life. Which of the specific schemas or shortcuts above do you think you might have used? Please explain

5. What kind of schemas do you think might have been involved in your sexual offending?

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Homework: Cognitive Distortions (Homework 3.3.2)

List and describe the *Cognitive Distortions* that 1) played a role in your sexual offense and 2) impacted other areas of your life. Also, if you have been able to challenge these distortions, please include this information as well.

1. The thinking errors that played a role in my sexual offending behavior included:

2. The thinking errors that played a role in other bad decisions I have made included:

3. If I was back in these situations again, how would I perceive them now?

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MODULE 4: Relationships and Intimacy

Key Learning Points/Module Goals

- Review attachment theory and understand how it applies to relationships/intimacy.
- Enhance understanding of love languages.
- Review essential components of intimacy and enhance intimacy skills.

Homework Assignments

- Improving or Maintaining a Secure Attachment (Homework 3.4.1)
- My Love Language (Homework 3.4.2)
- Intimacy Enhancement (Homework 3.4.3)

Handouts

- Bartholomew's Dimensional Model of Attachment (Handout 4)

Facilitators, inform the group participants that attachment theory is an essential framework with which to understand intimate relationships. As noted in the core phase of the program, early attachments with parents or other caregivers provide the foundation for an individuals' model of relationship functioning. When parents are loving, supportive, and available, a child learns that he has the characteristics that other people find loveable and that others are capable of giving love. In contrast, if parents are physically, sexually, emotionally abusive, or indifferent, the child receives negative messages about him/herself and relationships. Moreover, observing abusive behaviors between caregivers can have similar effects. It is in this context of early attachment relationships that children learn (or fail to learn) some of the more crucial most crucial aspects of social relationships. As an example, if children are included in discussions with their parents, they acquire conversational skills and learn how to read emotions. In contrast, if they are excluded from these interactions, they will fail to acquire such skills.

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Attachment Theory

Prior to presenting the core tenets of attachment, it is useful to ask the group members first in order to understand the information that was retained from the core phase of the program.

Attachment is described as a long-lasting psychological connection with a meaningful person that caused pleasure while interacting and soothed in times of stress. The quality of attachment has a critical effect on development and has been linked to various aspects of positive functioning, such as psychological well-being. It has been defined as lasting psychological connectedness between human beings and is similar in concept to an emotional bond. Attachment is often established during infancy with the primary caregiver and are further formed through ongoing "attachment transactions," defined as a continuing process of seeking and maintaining a certain level of proximity to another individual.

The attachment styles presented in the core phase of the program were based on Bowlby's attachment theory (1988) and were tested with the strange situation paradigm.

Secure Attachment

Bowlby (1988) described secure attachment as the capacity to connect well and securely in relationships with others while also having the capacity for autonomous action, as situationally appropriate. Secure attachment is characterized by trust, an adaptive response to being abandoned, and the belief that one is worthy of love. Secure adults tend to hold positive self-image and positive image of others, meaning that they had both a sense of worthiness and an expectation that other people were generally accepting and responsive. Notably, many secure adults may, in fact, experience negative attachment-related events, yet they are able to objectively assess people and events and assign positive value to relationships in general.

Anxious Attachment (some call this "Preoccupied")

Anxious attachment (also called ambivalent) relationships are characterized by a concern that others will not reciprocate one's desire for intimacy. As adults, those with an ambivalent attachment style are overly concerned with the uncertainty of a relationship. Individuals with this attachment style hold a negative self-image and a positive image of

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others, meaning that they have a sense of unworthiness but generally evaluated others positively. As such, they strive for self-acceptance by attempting to gain approval and validation from their relationships with significant others. They also require higher levels of contact and intimacy from relationships with others.

Avoidant Attachment (some call this "Dismissive")

A dismissive-avoidant attachment style is demonstrated by adults who hold a positive self-image and a negative image of others. They prefer to avoid close relationships and intimacy with others in order to remain a sense of independence and invulnerability.

Dismissive-avoidant adults deny experiencing distress associated with relationships and downplay the importance of attachment in general, viewing other people as untrustworthy.

Fearful Attachment

Individuals with a fearful avoidant attachment style have characteristics of both anxious and avoidant individuals. Bartholomew and Horowitz (1991) write that they tend to have negative views of both themselves and others, feel unworthy of support, and anticipate that others will not support them. As a result, their behavior may be confusing to friends and romantic partners; they may encourage closeness at first and then emotionally or physically retreat when they start to feel vulnerable in the relationship.

A similar, yet more simplified, dimensional model of attachment was formulated by Bartholomew and Shaver (1998) and is particularly applicable to adult relationships. The four types are differentiated based on ones' view of themselves and others.

Facilitators, provide Handout 4 (Bartholomew's dimensional model of attachment) and review the characteristics of each type. Ask participants the following:

1) What kinds of childhood experiences might lead to different adult attachment styles?
2) What type of attachment style do you present with:
    a. During the time of the offense

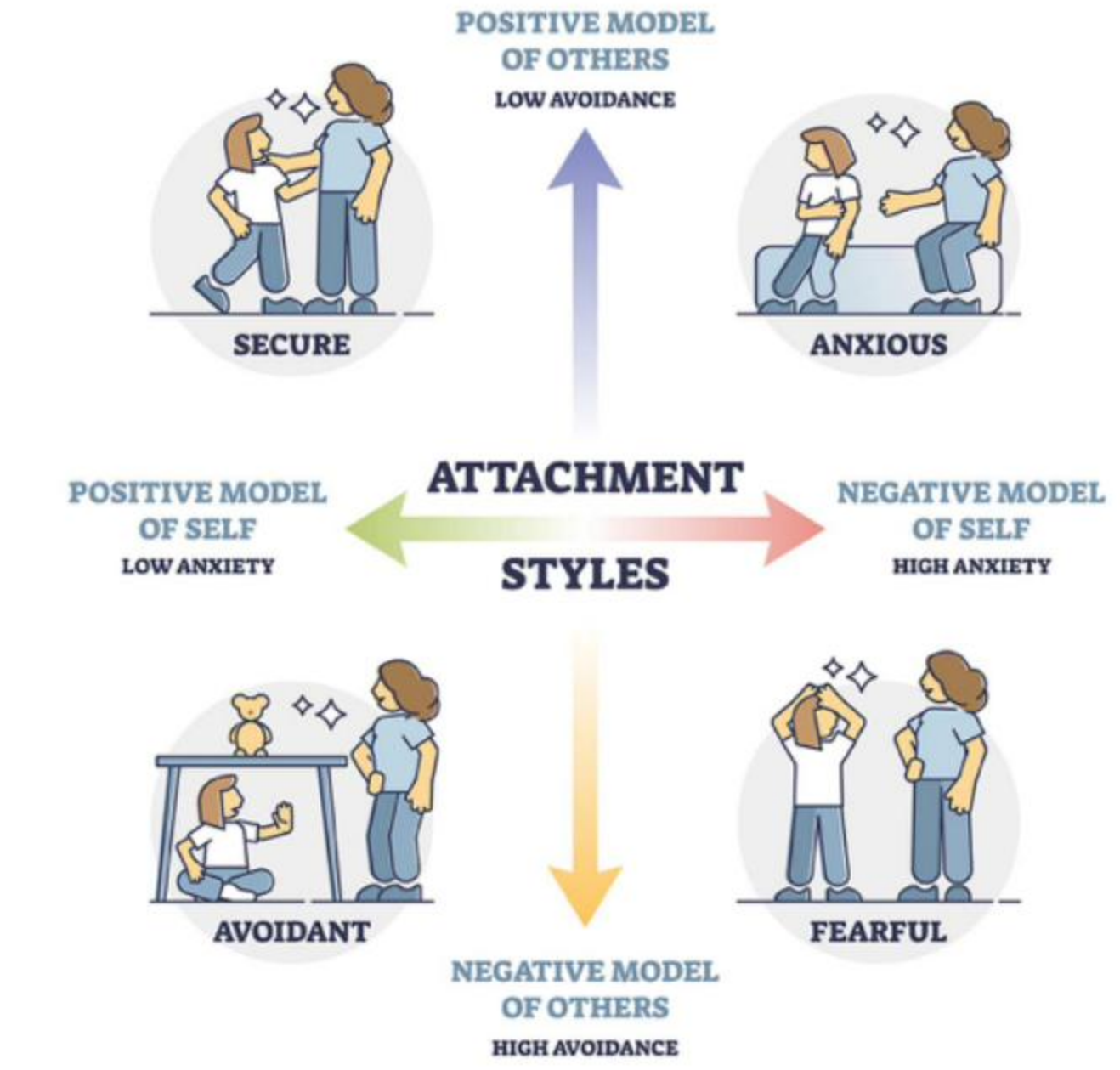
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b. Currently

Compare the results of the discussion with the results of their attachment style quiz administered in the core program (Homework 2.5.2). Are these results consistent with the discussion today?

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Handout 4: Bartholomew's Dimensional Model of Attachment



Characteristics of Adult Attachment Styles (*www.simplypsychology.org*)

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Homework: Improving or Maintaining a Secure Attachment (Homework Assignment 3.4.1)

Have the group members identify and implement plans for more secure attachments. Many of these skills were discussed in the core program, so hopefully most group members can speak to how they have been implementing these skills over the last several months/years. Please refer to Block II for a more exhaustive list of ideas, but examples include:

- Prioritizing personality characteristics rather than physical attributes of potential partners
- Not equating sex with love or intimacy
- Becoming more emotionally expressive
- Working on issues around trust

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Homework: Improving or Maintaining a Secure Attachment (Homework Assignment 3.4.1

Describe the ways you have attempted to develop or maintain a secure attachment. If you have not implemented any plans, what are some ideas that you will be implementing in the future to ensure a more secure attachment style.

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Love Languages

According to a theory by Gary Chapman (1995), each person has one primary and one secondary love language. Facilitators, present the love languages to the group, although ask the members to see if they can recall the key elements of the different types.

Language 1: Words of Affirmation

One way to express love emotionally is to use words. Verbal compliments, or words of appreciation, are powerful communicators of love. They are best expressed in simple, straightforward statements of affirmation, such as:

- "You look really nice in that suit."
- "Do you ever look incredible in that dress! Wow!"
- "I really like how you're always on time to pick me up at work."
- "You can always make me laugh."

Words of affirmation are one of the five basic love languages. Within that language, however, there are many dialects. All of the dialects have in common the use of words to affirm one's spouse. Psychologist William James said that possibly the deepest human need is the need to feel appreciated. Words of affirmation will meet that need in many individuals.

Language 2: Quality Time

Quality time refers to giving someone your undivided attention. This does not mean sitting on the couch watching television together. When you spend time that way, Netflix or HBO has your attention — not your spouse. What I mean is sitting on the couch with the TV off, looking at each other and talking, devices put away, giving each other your undivided attention. It means taking a walk, just the two of you, or going out to eat and looking at each other and talking. Time is a precious commodity. We all have multiple demands on our time, yet each of us has the exact same hours in a day. We can make the most of those hours by committing some of them to our spouse. If your mate's primary love language is quality time, she simply wants you, being with her, spending time.

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Language 3: Receiving Gifts

Almost everything ever written on the subject of love indicates that at the heart of love is the spirit of giving. All five love languages challenge us to give to our spouse, but for some, receiving gifts, visible symbols of love, speaks the loudest. A gift is something you can hold in your hand and say, "Look, he was thinking of me," or, "She remembered me." You must be thinking of someone to give him or her a gift. The gift itself is a symbol of that thought. It doesn't matter whether it costs money. What is important is that you thought of him or her. It is not the thought implanted only in the mind that counts, but the thought expressed in actually securing the gift and giving it as the expression of love. But what if the person says, "I'm not a gift giver. I didn't receive many gifts growing up. I never learned how to select gifts. It doesn't come naturally for me." Congratulations, you have just made the first discovery in becoming a great lover. You and your spouse speak different love languages. Now that you have made that discovery, get on with the business of learning your second language. If your spouse's primary love language is receiving gifts, you can become a proficient gift giver. In fact, it is one of the easiest love languages to learn.

Language 4: Acts of Service

Consider actions such as cooking a meal, setting a table, emptying the dishwasher, vacuuming, changing the baby's diaper, picking up a prescription, keeping the car in operating condition — they are all acts of service. They require thought, planning, time, effort, and energy. If done with a positive spirit, they are indeed expressions of love.

A willingness to examine and change stereotypes is necessary in order to express love more effectively. Remember, there are no rewards for maintaining stereotypes, but there are tremendous benefits to meeting the emotional needs of your spouse. If your spouse's love language is acts of service, then "actions speak louder than words."

Language 5: Physical Touch

We have long known that physical touch is a way of communicating emotional love. Numerous research projects in the area of child development have made that conclusion: babies who are held, stroked, and kissed develop a healthier emotional life

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than those who are left for long periods of time without physical contact. Physical touch is also a powerful vehicle for communicating marital love. Holding hands, kissing, embracing, and sexual intercourse are all ways of communicating emotional love to one's spouse. For some individuals, physical touch is their primary love language. Without it, they feel unloved. With it, their emotional tank is filled, and they feel secure in the love of their spouse. Implicit love touches require little time but much thought, especially if physical touch is not your primary love language and if you did not grow up in a "touching family." Sitting close to each other as you watch your favorite television program requires no additional time, but may communicate your love loudly. Touching your spouse as you walk through the room where he is sitting takes only a moment. Touching each other when you leave the house and again when you return may involve only a brief kiss or hug, but will speak volumes to your spouse.

Homework: My Love Language (Homework Assignment 3.4.2)

Group members are to answer the following questions:

1. What is my primary and secondary love language?
2. Provide examples in either your most recent relationship or your current relationship (if applicable).

Facilitator Tip – Compare the results of this homework assignment to the results of their presentation during the core phase of the program. Was it consistent or did they perceive themselves as manifesting a different love language? Why might this be the case?

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Homework: My Love Language (Homework Assignment 3.4.2)

Please respond to the following questions. If you have never been in a relationship before, please respond to question 1 with respect to what you think your love language will be in a future relationship.

1. What is my primary and secondary love language? What is my partners love language (if applicable)?

2. Provide examples of this love language in either your most recent relationship or your current relationship (if applicable).

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Intimacy

As discussed in Block II, intimacy is an important factor in relationships, and people who have committed sexual offenses often score low on measures of intimacy. In fact, "intimacy deficits" have been associated with feelings of loneliness and re-offending. In fact, the Good Lives Model, which was discussed in Module 1 of this maintenance curriculum includes intimacy as a primary human good (i.e., relatedness).

Facilitators, ask the group members to define intimacy and why it is important. Responses should be written on the white board and consistent themes underlined. The discussion should emphasize that intimacy is a bond that is based on trust, love, respect, and sharing. Intimacy provides emotional support, comfort, and security, particularly when experiencing stress or some other form of negative affect. Of course, intimacy characterizes not only romantic relationships, but also friendships as well. Intimacy is also associated with better psychological and physical health. Some review of the types of intimacy (e.g., physical, emotional, intellectual, experiential, and spiritual) may be helpful here (see Block II).

Homework: Intimacy Enhancement (Homework Assignment 3.4.3)

Have group members respond to two questions:

1) What have I done since Block II to enhance intimacy?

2) How am I going to build on this and further enhance intimacy over the next few months and years?

Group members are expected to present this assignment to the group and be prepared to receive feedback from the group and answer questions. Given the higher expectations of the maintenance program, many individuals will re-do this assignment based on the feedback and questions from the group. At minimum, a good assignment should reflect aspects of communication styles, mutual respect and caring, empathy, shared interests and compromise, love languages, etc. It is also helpful to present the information pertaining to sustaining a

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healthy relationship from Linehan (2014) – GIVE (DEAR MAN GIVE) also presented in more detail in Block II.

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Homework: Intimacy Enhancement (Homework Assignment 3.4.3)

Respond to the following questions regarding intimacy:

1. What have I done since Block II to enhance intimacy in my relationships?

2. How am I going to build on this and further enhance intimacy over the next few months and years?

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MODULE 5: Healthy Sexuality

Key Learning Points/Module Goals

- Understand the essential features of healthy sexuality.
- Understand and manage unhealthy sexual behaviors.

Homework Assignments

- Healthy Sexuality (Homework 3.5.1)
- Meeting our Sexual Needs in Healthy Ways (Homework 3.5.2)
- Managing Unhealthy Sexual Behavior (Homework 3.5.3)

Handouts

- None

Health sexuality is an important goal in this program and is instrumental in living a good life. The facilitator should write "**Healthy Sexuality**" on the board and ask participants to define it.

Facilitator Tip – It would be helpful to review past progress notes from Module 11 in the core program to determine if their views have changed since this was initially discussed. Hopefully, the group is able to identify the following:

According to the World Health Organization (WHO, 2006), the current working definition of sexual health is as follows:

> ...a state of physical, emotional, mental and social well-being in relation to sexuality; it is not merely the absence of disease, dysfunction or infirmity. Sexual health requires a positive and respectful approach to sexuality and sexual relationships, as well as the possibility of having pleasurable and safe sexual experiences, free of coercion, discrimination and violence. For sexual health to be attained and maintained, the sexual rights of all persons must be respected, protected and fulfilled.

https://www.who.int/reproductivehealth/topics/sexual_health/sh_definitions/en/

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Of course, one of the key definitions is consent, and this should be reviewed with the group members in detail. At its core, consent is an agreement between participants to engage in sexual activity. Consent is when someone agrees, gives permission, or says "yes" to sexual activity with other persons.

CERTS Model

Experimental Exercise: Write out the core elements of the CERTS model and generate discussion on each component prior to writing and discussing the definition with examples.

> **CONSENT** means you can freely and comfortably choose whether or not to engage in sexual activity. This means you are conscious, informed, and able to stop the activity at any time during the sexual contact.
>
> **EQUALITY** means your sense of personal power is on an equal level with your partner. Neither of you dominates or intimidates the other.
>
> **RESPECT** means you have positive regard for yourself and for your partner. You also feel respected by your partner based on how your partner is treating you.
>
> **TRUST** means you trust your partner on physical and emotional levels. You accept each other's needs and vulnerabilities and are able to respond to concerns with sensitivity.
>
> **SAFETY** means you feel secure and safe within the sexual setting. You are comfortable with and assertive about where, when, and how the sexual activity takes place. You feel safe from the possibility of negative consequences, such as unwanted pregnancy, sexually transmitted infection, and physical injury.

Homework: Healthy Sexuality (Homework Assignment 3.5.1)

Facilitators are to provide Homework 3.5.1 on healthy sexuality. It is important to ensure clients understand these two questions and are prepared to provide sufficient detail. Again, many are required to redo this assignment based on feedback from the group. With regard to question #2, it is important that they demonstrate an understanding of what is healthy vs unhealthy sex and

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how best to communicate their desires to their partner. Role-play exercises are helpful to build and maintain this skill. A review of sexual myths presented in the core program may also be helpful.

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Homework: Healthy Sexuality (Homework Assignment 3.5.1)

Please provide a detailed answer to these two questions related to healthy sexuality:

1. Describe how you have implemented the CERTS model in your current relationship by providing specific examples. If you are not in a current relationship, describe how you will implement the CERTS model.

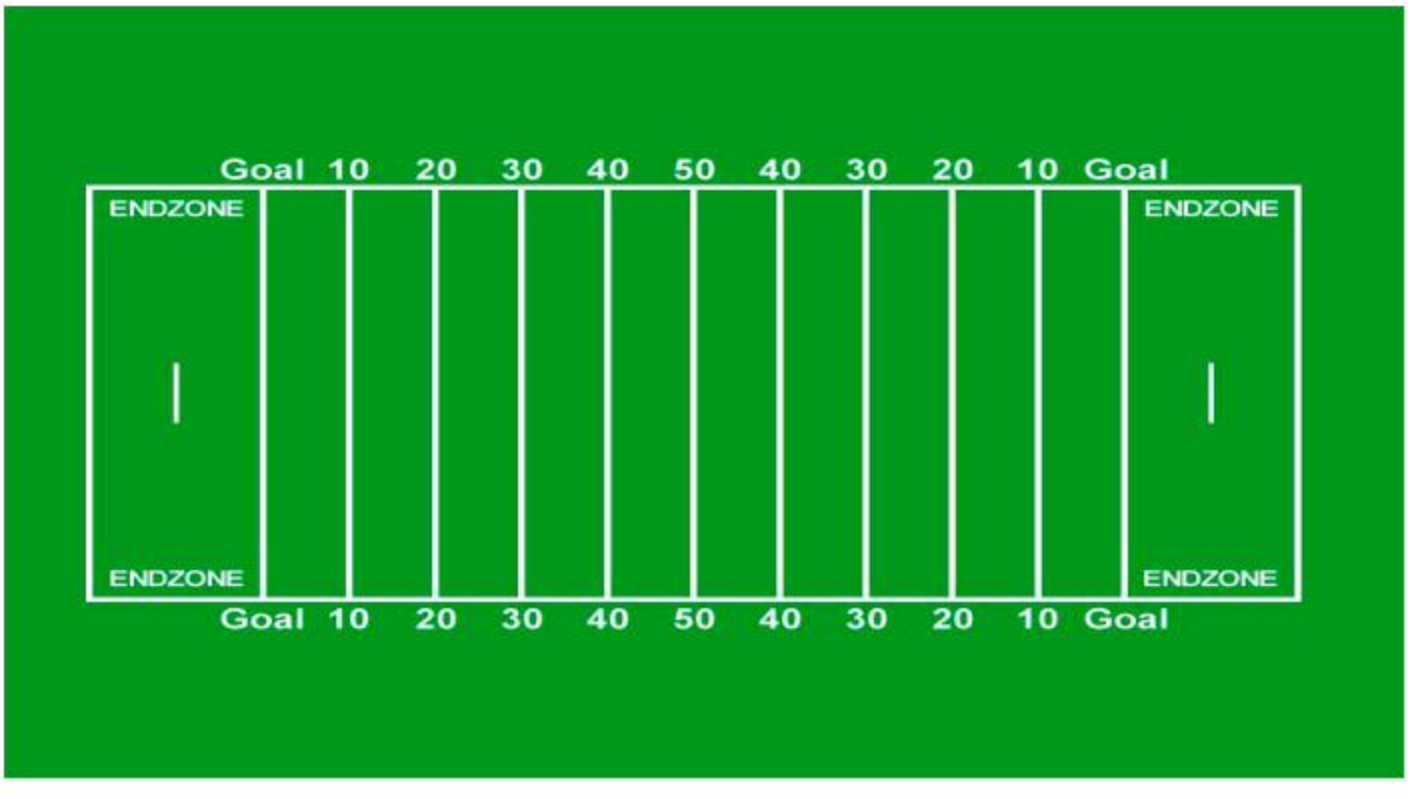
2. What are the steps you have taken (or will take) to ensure you are meeting your sexual needs in healthy ways and being responsive to your partner?

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Which Sexual Behaviors are Healthy?

The CERTS model describes an essential feature of healthy sexuality; that is, consent. Participants should recall the football analogy in Module 11. It may be helpful to remind them of this example. Essentially, every sexual thought, feeling, urge or behavior can be placed somewhere on the football field. In this example, let's pretend that a healthy sexual thought or behavior brings us closer to the "home team end zone" and an unhealthy thought brings us closer to the "visitor team end zone." A though that made it more likely to result in a healthy sexual relationship (e.g., I need consent to move forward with this sexual activity) would be closer to the home team end zone, whereas an unhealthy thought (e.g., she is drunk so obviously was ok with this at some point) would be closer to the visitor team end zone.

Experiential Exercise: Draw the following football field on the board and have participants identify thoughts or behaviors of theirs that would be closer to one end zone and thoughts or behaviors that were closer to the other end zone. It is important to emphasize that everyone has had thoughts or behaviors that spanned the entire field at one point or another so it is important to provide examples of both.



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A group discussion should follow that identifies the list of sexual behaviors that are considered unhealthy versus healthy. Some examples may include masturbation, sex in the missionary position, and anal sex. Ask group members which of these behaviors they consider to be appropriate and ask them to justify their decisions. This discussion typically reveals three categories of activities; appropriate, alternative/unusual, and deviant (i.e., harmful or unhealthy). Generate a discussion as to the basis for which they make these determinations. These discussions should tie in the previous information regarding components of healthy sexuality. Research has shown that many individuals who have committed sexual offenses have a distorted and somewhat prudish attitude toward sex. In other words, any sexual practices other than missionary position are considered unusual or even harmful. The purpose of this discussion is to provide some psychoeducation on the prevalence of other types of sexual activity (e.g., anal sex) and how these are not relatively unusual or even harmful as long as the components of CERTS are met. In fact, having larger discussions about how individuals can speak to their partners regarding their own sexual desires and interests while respecting that their partners may not share those interests are healthy and can reduce risk of sexual recidivism.

Homework: Meeting our Sexual Needs in Healthy Ways (Homework Assignment 3.5.2)

Based on the above, participants should indicate how they have improved their sexual thoughts and behaviors with their partners (if they have one), or how they intend to do this with their future sexual partners. Examples include ways that they are communicating with their partners about their desires or how they ask their partners what they desire. This homework assignment was completed in Module 11, so it will be important to discuss how this skill has improved since the core program.

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Homework: Meeting our Sexual Needs in Healthy Ways (Homework Assignment 3.5.2)

Please indicate how you have improved your sexual thoughts and behaviors with your partner since beginning this program. If you do not have a partner currently, how will you be different when you start a romantic relationship with someone. Examples may include ways that you communicate with your partner about their desires or how you ask them what they desire.

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Risk Factors: Unhealthy Sexuality

In the context of healthy sexuality, it makes sense to review the following risk factors that are associated with unhealthy sexuality as well as re-offending: Sexual Preoccupation, Sex as Coping, and Deviant Sexual Interests. Perhaps some of these were noted during the previous exercise.

The facilitator should ask participants to define each risk factor and why they are problematic.

Sexual preoccupation is a construct that has many names, such as sexual addiction, hypersexuality or compulsive sexual behavior disorder. Regardless of what we call it, it highlights two main components: *sex drive* and *problematic sexuality*. Sex drive includes sexual frequency and preoccupation, such as number of sexual partners and how much one thinks about sex, whereas problematic sexuality includes impulsivity and using sex to cope with negative emotional states.

With regard to deviant sexual interests, group members should be reminded about the paraphilias and the ones that are found more commonly among people who have committed sexual offenses.

Paraphilias are defined in the latest version of DSM as recurrent and intense sexual arousal to atypical objects or activities, as manifested in sexual fantasies, urges, or behavior over at least a six-month period (American Psychiatric Association [APA], 2013). In the DSM-5, paraphilias are distinct from paraphilic disorders; the latter term denotes a paraphilia that is accompanied by distress or impairment in functioning. Paraphilias are necessary, but not sufficient, for determining the presence of a paraphilic disorders (APA, 2013). Eight paraphilias are specifically listed in the DSM-5: pedophilia (prepubescent children), exhibitionism (exposing one's genitals to an unsuspecting stranger), voyeurism (spying on unsuspecting strangers in normally private activities), sexual sadism (inflicting humiliation, bondage, or suffering), sexual masochism (experiencing humiliation, bondage, or suffering), frotteurism (touching/rubbing against a non-consenting person), fetishism (non-genital body parts or nonliving objects), and

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transvestism (cross-dressing). Many other paraphilias have been described in clinical case studies.

Paraphilic disorders are most commonly seen in forensic and correctional settings, when paraphilic behaviors are acted upon (if illegal), such as accessing child sexually explicit material or having sexual contact with a child (Seto, 2008, 2013). Other paraphilias that are commonly seen in forensic and correctional settings include exhibitionism, voyeurism, and sexual sadism.

It is important to discuss with clients that paraphilias, particularly pedophilia, are seemingly innate, meaning that it is a biologically based orientation. The important message here is that **we don't choose our sexual interests, but we do choose whether or not we act on them.**

Experiential Exercise: Ask group members to discuss whether or not they have experienced problems managing their sexual urges or behaviors or using sex to cope or if they struggle with atypical sexual interests.

Facilitator Tip – It is useful to review their previous homework assignment in Module 13 - Identifying My Level of Sexual Desire/Urges.

Homework: Managing Unhealthy Sexual Behaviors (Homework Assignment 3.5.3)

Group members should respond to the following question:

How have I managed unhealthy sexual behaviors and how will I maintain healthy sexual behavior? Of course, some participants will not struggle with these issues, but they should still respond to how they might manage such issues in the future (e.g., maintain healthy coping to prevent the likelihood of using sex to cope). This assignment should include some relevant CBT and DBT skills, such as relaxation, the STAR method, and cognitive restructuring. The Relapse Prevention model can sometimes be a helpful guide here.

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Homework: Managing Unhealthy Sexual Behaviors (Homework Assignment 3.5.3)

Please respond to the following question:

1. How have I managed unhealthy sexual behaviors in the past?

2. How will I manage unhealthy sexual behavior in the future?

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MODULE 6: Self-Management Plan

Key Learning Points/Module Goals

- To review the offense chain and ensure the clients have a solid understanding of the factors associated with their sexual offense.
- To be aware of their risk factors and solidify a coping plan.
- To integrate much of the previous curriculum into an overall plan for the future.

Homework Assignments

- Risk Factors and Coping (Homework Assignment 3.6.1)
- Behavioral Progression (Homework Assignment 3.6.2)
- Self-Management Plan (Homework Assignment 3. 6. 3)

Handouts

- Relapse Prevention Offense Chain (Handout 5)
- Self-Regulation Model (Handout 6)
- Behavioral Progression (Handout 7)

In this module, group members integrate what they have learned throughout the curriculum to solidify a healthy plan for the future. Given that this is the final module, clients are expected to present with a considerable amount of understanding and insight. It is not uncommon to have clients re-write the assignments based on feedback and present it again to the group.

At this point in the curriculum, group members should have a solid understanding that sexual offending does not just happen, such that there are always factors that occur prior to the decision to engage in problematic sexual behaviors.

Present Handouts 5 and 6. Both the relapse prevention model and self-regulation models are descriptions of the specific offense chain. In addition to ensuring each group member has a solid understanding of each model, discussions should take place on how everyone relates to

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these models. Individuals should have a solid understanding of the key components such as high risk situations, seemingly unimportant decisions, and lapses, for relapse prevention and the four pathways (i.e., avoidant-passive, avoidant active, approach-automatic, approach-explicit) of the self-regulation model. Which elements resonate with them? Which aspects do they feel relate specifically to their prior offense(s)?

Homework: Risk Factors and Coping (Homework Assignment 3.6.1)

In this assignment, group members describe the feelings and thoughts that could put them at risk for offending again. This should include the events or situations where one could be at risk. For each risk factor identified, ensure that a specific coping skill is provided. The assignment is typically presented by the group member, and after positive comments and praise, the group proceeds to ask questions or provide feedback.

Facilitator Tip – Some clients may indicate that they are not at risk or that there is no situation that would pose a risk to them moving forward. It is important to reiterate that no one presents with zero risk and that it is those individuals who acknowledge some risk (even if minor) who tend to excel in life. A helpful analogy is driving a car. We can and should obey traffic laws, wear a seat belt, have a valid driver's license, etc., but there is always a risk of getting into a car accident. At the very least, we should be aware of the possibility of putting ourselves in a situation where we can be accused of an offense. While it may not seem fair, those individuals who have a prior criminal record have to be particular vigilant about ensuring they are adhering to their good life plan.

Facilitator Tip – These are the aspects that make a good risk factors assignment. Of course, all things on this list do not need to be in the assignment, although at this stage in the curriculum, many of these items should be present. However, inclusion of any of these items is worthy of praise. After the question and feedback portion, this assignment is typically re-written so that it adequately reflects all of the feedback from the group (and you).

What Makes a Good Risk Factors Assignment?

- A variety of risk factors – internal and external
- Thoughts, feelings, events, and behaviors related to risk

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- Some depth to their understanding of risk factors – how they can build upon one another
- Accuracy – some degree of consistency with what they see as risk factors and what you and other group members see
- Acceptance of the need for future awareness and vigilance
- Recognition of the impact of coping styles and the use of adaptive coping (i.e., predominantly task-focused coping)

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Homework: Risk Factors and Coping (Homework Assignment 3.6.1)

1. Please describe the events or situations that could put you at risk:

2. Please describe the thoughts and feelings that could put you at risk:

3. What are the main coping skills that I will use to handle these risk factors?

Homework: Behavioral Progression (Homework Assignment 3.6.2)

Facilitators should provide clients with the behavioral progression handout (handout 7) and illustrate it on the board to aid discussion. Ensure that clients understand the main components of the model.

Distal problems or background factors are the areas of one's life that make someone vulnerable to committing a sexual offense. This can include factors in early life or more recent attitudes and beliefs. Triggering events or life events are factors that tend to start the behavioral progression; that is, the path toward a sexual offense. Of course, the impact of the triggering event is often influenced by other factors, such as our interpretation of that event. The distress factor speaks to how the individual responded to the triggering event. Typically, some emotional distress is present, whether it is anger, frustration, loneliness, or some other emotional experience. A maladaptive response describes how the client reacted to the triggering event and their initial distress. As noted in the handout, this could include substance use, intense emotional states, or maladaptive attitudes. Finally, there is the offense.

Facilitator Tip – It would be helpful to integrate elements of the self-regulation model (e.g., goals and strategies with respect to offending and what type of post-offense evaluation they had) as well as relapse prevention (e.g., high risk situations) in your discussions (see handouts 5 and 6).

Once everyone has a good understanding of the model and has provided some input, the clients are ready to be assigned the behavioral progression homework assignment. Given the depth of this assignment, no more than one or two clients should present, as it is important to leave room for feedback and questions. Depending on the feedback, clients often redo this assignment so that they can incorporate the helpful feedback provided.

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Homework: Behavioral Progression (Homework Assignment 3.6.2)

Please respond to the following:

1. What background factors (or distal problems) contributed to my sexual offense?

2. What triggering event (or life event) started the specific pathway to my offense?

3. What was my maladaptive response to the triggering event?

4. How did I feel after I committed my offense?

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Homework: Self-Management Plan (Homework Assignment 3.6.3)

In this final assignment, group members reflect on everything they have learned in the entire program across all phases and modules. Individuals are to write out a plan for how they are going to manage their life differently and more positively in the future. Of course, some elements of the previous exercise can be included (i.e., risk factors and coping), but emphasis should also be placed on their goals for the future. For those nearing their discharge date, these goals should reflect their life after parole. For those who are not nearing their discharge date, the goals can reflect life after they transition to the aftercare program at HOPE.

A good self-management plan should incorporate elements that are going to reduce risk as well as promote a healthy, good life. A good question to start this process is to ask them to visualize themselves 1 year or 5 years after parole (if relevant). How would they be different. Be specific about what this person looks like and how do they achieve it.

What makes a good self-management plan?

- Solution-focused coping strategies
- The identification of external support systems (family and friends)
- Achievable and realistic goals and expectations
- References to strengths that the individual will build upon
- Approach-oriented goals
- Choosing appropriate living companions and location
- Sensible work plans that are attainable and free from access to potential victims

Facilitator Tip – Although everyone's self-management plan will be different, the above noted features are generally a sign of a good plan.

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Since this is an open-ended and less-structured assignment that requires some integration of previous material discussed, ensure that clients have a good understanding of what this assignment entails and overall expectations.

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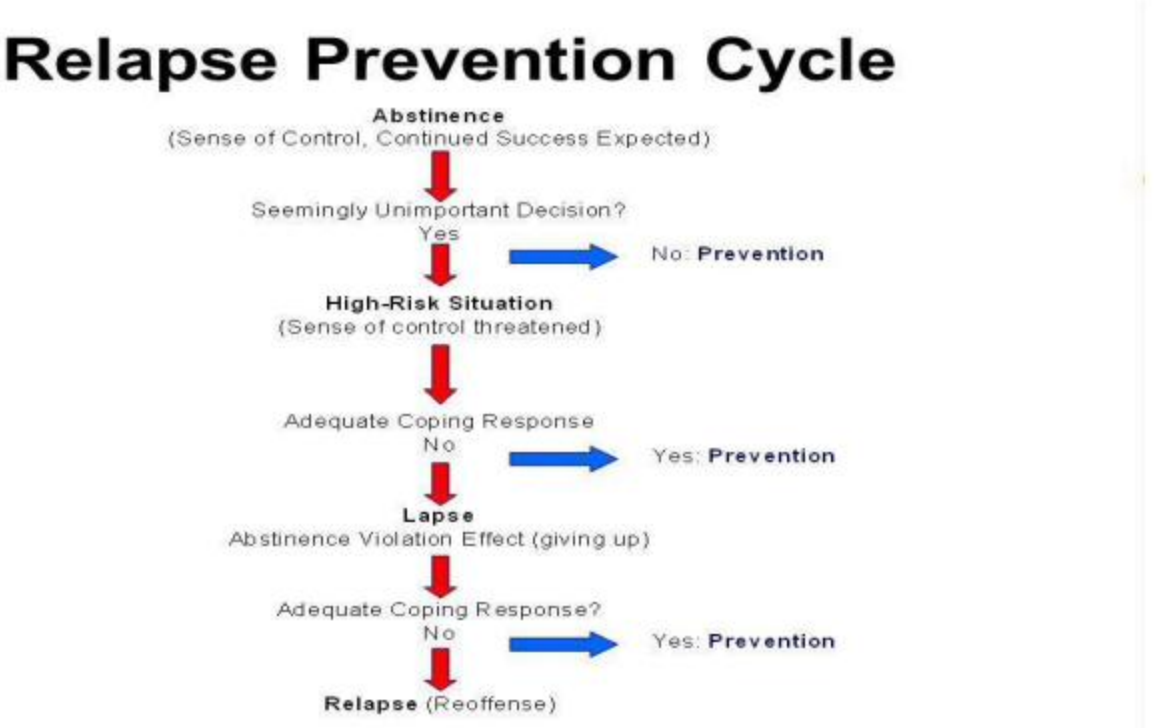
Homework: Self-Management Plan (Homework Assignment 3.6.3)

Please describe your overall plan for a healthy life. It may be helpful to start with the question:

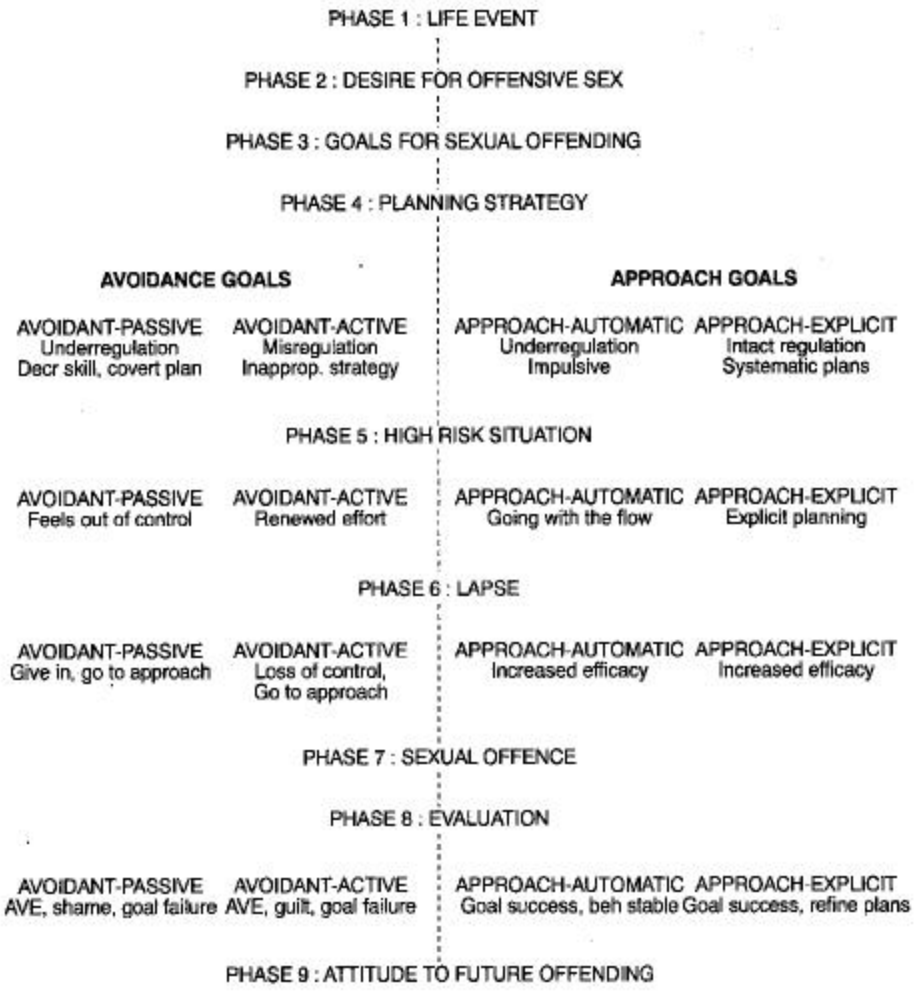
1. Who do I want to be in the future (e.g., 5-years from now). What is my plan to get there and how do I ensure I will never commit another sexual offense or put myself in a position to be accused of one?

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Handout 5: Relapse Prevention Offense Chain

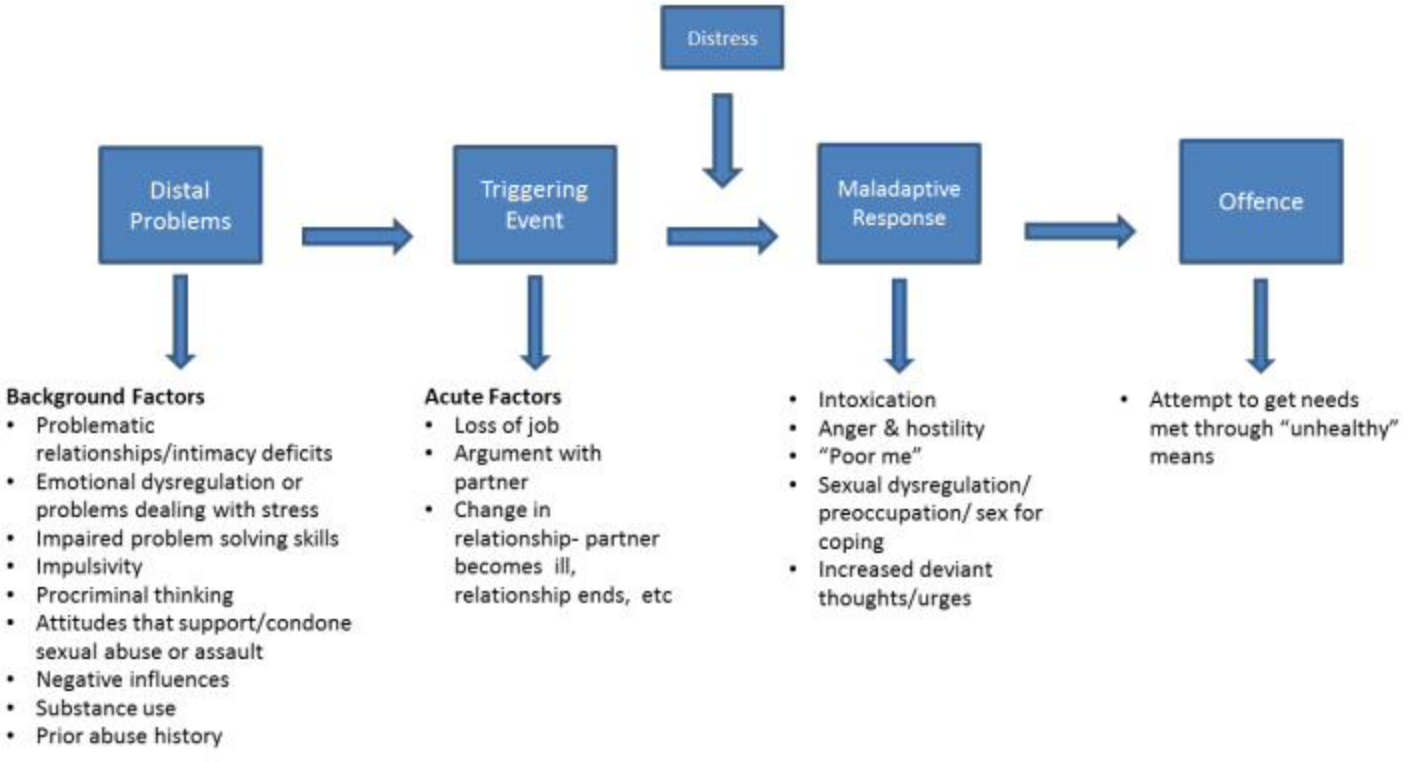


Handout 6: Self-Regulation Model



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Handout 7: Behavioral Progression



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